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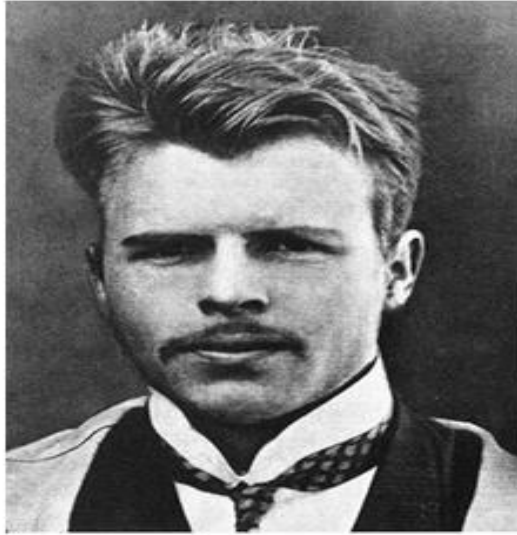
A Guide to Modified Scoring System

Dr Rakesh Kumar

Practical Tips & Skills for Beginners

RORSCHACH INKBLOT TEST:

A Guide to Modified Scoring System



Hermann Rorschach

Dr. Rakesh Kumar

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Agra (India)



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FOREWORD

I have much pleasure in writing a brief introduction to Dr. Rakesh Kumar's admirable efforts in writing a book under name and style of "*Rorschach Inkblot Test: A Guide to Modified Scoring System*", for the students and professional psychologists in India and Asian countries.

Two words in whole of the psychology have acquired popularity amongst general public and Health professionals. These two words are IQ and Rorschach. IQ is synonymous to psychology and Rorschach is synonymous to clinical psychology. Clinical psychology has become a popular professional branch of psychology [also as an important branch of Rehabilitation Sciences by the act of Parliament in India], over a period of time, all over the World, so is the Rorschach. In a national survey in the USA; the Rorschach was ranked VIII among psychological tests used in mental health facilities. It is the second most widely used test by the member of the Society for Personality Assessment and it is requested in 25% of forensic assessment cases. Weiner, I.B. and Greene R.L. [2007] in their book 'Handbook of Personality Assessment' said that in a survey, 80% of clinical psychologists engaging in assessment services utilize the Rorschach, and same percentage of psychology graduate programs teach it.

Almost nine decades back, the Rorschach test was made available. Books that are available on this important clinical/psycho-diagnostic test were written and published in Western countries, printed in volumes, consisting several hundred of pages and are not easily available, in addition to their high procurement price in India; forbidding students to have a personal copy as a ready reckoner in

their clinic/psychological laboratory. No efforts have ever been made to provide an easy, simple, practical, affordable source of learning in India and Asian countries for the students of clinical psychology on this important psycho-diagnostic test.

Nevertheless, there are four publications from India on Rorschach. [1] Rorschach Psycho-diagnostics: An Indian Case Study by C.L. Kundu, published by Vishal Publication, 1980; [2] A Pragmatic View of Rorschach Inkblot Technique by B.L. Dubey, published by National Psychological Corporation, 1982; [3] Rorschach Test: Theory and practice by R.K. Mishra and others, published by Sage Publications, 1996; and [4] The Rorschach Test [A Practical Manual] by Dwarka Pershad and S. Parekh; published by H.P. Bhargava Book House, 2001. The first two books are research monographs [published Ph.D. work] and are not useful for the students. The third book was not written for the students and learner. The fourth book 'The Rorschach Test [a practical manual]' may be considered as first ever effort to provide a simple, concise affordable book to the learner. This book, however, is in text book style.

In this hierarchy, Dr. Rakesh Kumar's effort in writing a book 'in class notes fashion', describing difficult concepts in points, giving illustrations for easy grasp, using examples from Indian patients and providing master's rating to the exercises for practice, is a laudable effort. With his long standing in the profession as a teacher, researcher, writer, trainer and practitioner, he has developed confidence and clarity on various difficult concepts of Rorschach related to its administration, scoring, report writing and interpretation that he has amply reflected in his book. I am sure Dr Rakesh Kumar's book "*Rorschach Inkblot Test: A Guide to Modified Scoring System*" will interest the learners to acquire skill and confidence in the art of using Rorschach psycho-diagnostic test.

Chandigarh

Independence Day, 2009

Dwarka Pershad

PREFACE

The Book ***“Rorschach Inkblot Test: A guide to modified scoring system”*** written by Dr. Rakesh Kumar, Coordinator, Department of Clinical Psychology, Institute of Mental Health and Hospital, Agra; is though intended to address the need of the beginners in psychology, I find that it should be equally helpful for teachers of clinical psychology and psycho-diagnostic assessment. The book is written and organized in such a manner that it can also prove to be a good resource and ready reference manual for various mental health professionals interested in the use of inkblot techniques in diagnostic assessment.

As a teacher of clinical psychology, I have always seen students wondering in search of an applied and easily comprehensible manual that can help them to understand and assimilate the art of administration, scoring and interpretation of the Rorschach technique; I believe that the book by Dr. Rakesh will definitely fill this gap. This manual will help students not only to understand the basic principles of the scoring and interpretation of the Rorschach Inkblot Test but will also facilitate the development of the clinical skills of formulating a diagnosis through this test.

The literary presentation of the book is very simple and itemized to facilitate understanding of the complex procedures. The author has tried to include almost all the essential aspects needed for developing the skill in using the Rorschach Inkblot Test. Everything required to learn the art of using Rorschach for diagnostic purpose is included in this manual, right from the rationale of inkblots to administration, scoring, interpretation and writing assessment report.

In my teaching career I have gone through several manuals and guidebooks for administration, scoring, and interpretation of the Rorschach, this one appears to me *the best I have ever seen*. There are several reasons why I claim so. It presents all essential details of the art of using the Rorschach Inkblot test in a nutshell. The book is well planned and written in a very lucid manner taking all the practical difficulties encountered by the beginners in learning the Rorschach. For example, at appropriate places the author has discussed (citing suitable examples) how to handle common difficulties encountered by new users of Rorschach in administration and scoring of the test. The most innovative and remarkable thing about this manual is that the author has provided

several general guidelines for taking correct decision in face of situations where subjectivity is likely to intrude.

I am of the opinion that it is a book that will prove fruitful as a teaching and learning aid for students and teachers as well.

Dr. Rakesh Pandey, Ph.D.

Course Coordinator, P.G. Diploma in Counseling and Psychotherapy

Editor, Indian Journal of Social Science Researches

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ACKNOWLEDGEMENT

This guide to modified scoring system for the most prestigious Rorschach Inkblot Test is addressed to the need of beginner postgraduate psychology students. This quick reference manual is based on my more than one decade of experience in training through workshops, lectures and individualized instructions to postgraduate students of psychology including those who were enrolled in postgraduate diploma courses in clinical psychology. The supervision of doctoral dissertations employing Rorschach Inkblot test and Somatic Inkblot test has also contributed significantly to the modifications introduced in this manual. The advanced students of Rorschach may quickly review the manual to familiarize with the scheme of scoring and interpretation presented in this volume.

It must be remembered that administration, scoring and interpretation of Rorschach Test involves rich understanding of psychopathology, theories of personality, a thorough understanding of the concepts used in Rorschach test and supervised training. No single manual can equip a professional to become an adept in the inkblot technique. So it is recommended that any person using this manual should get adequate supervised clinical training. This guide alone may not be used to arrive at final psychodiagnostic formulations.

I am grateful to Prof. Sudhir Kumar, Director & CEO, Institute of Mental Health and Hospital, Agra for providing me optimum opportunity to pursue my academic interests.

I express my deep sense of gratefulness to esteemed Dr. R.G. Varshney, Principal, Digamber Jain College, Baraut for allowing me to pursue supervision of doctoral dissertations on inkblots through the college.

I owe my deep gratitude to my teachers who taught me inkblot techniques: Dr. B.B.Tiwari, Prof. T.R.Shukla, Prof. Dwarka Pershad and Prof. B.L.Dubey. I also have a deep sense of gratitude for my teachers: Dr. B.S. Tripathi, Dr. Beer Singh, Dr. Jyotsna Sharma, Dr. Uma Rani, Dr. Rajni Kashyap, Dr. R.G.Sharma, Dr. D.K. Kenswar, Dr. Mamta Chaturvedi and Sri Ramesh Chander whose blessings always remain with me.

I am indebted to the faculty and students of following Institutes/Universities/colleges for providing me an opportunity to refine and update my skills in Rorschach Inkblot test: Central Institute of Psychiatry, Ranchi; Institute of Mental Health and Hospital, Agra; Dev Sanskriti Viswavidyalya, Hardwar; St. John's College, Agra; CCS University, Meerut ; Digamber Jain College,

Baraut; Purvanchal University, Jaunpur; Barkatullah University, Bhopal; Utkal University, Bhubaneswar; Banaras Hindu University, Varanasi; M. D. University, Rohtak; Lucknow University, Lucknow; Raghunath Girls College, Meerut; Amity University, Noida; Guru Jambheshwar University, Hisar and RBS College, Agra.

I wish to express my gratitude to Dr. G.K.Makhija, Prof. O.P.Mishra, Prof. I.L.Sharma, Prof. S.N.Rai, Prof. P.K. Mishra, Dr. D. Sahoo, Prof. Archana Shukla, Prof. T.B.Singh, Dr. Yashveer Singh, Prof. Nov Ratan Sharma, Prof. Ramjee Lal Srivastava, Dr. S.P.K. Jena, Prof. Bhupinder Singh and Prof. Beer Singh for their encouragements and support.

I gratefully acknowledge the contribution of my doctoral students who worked on inkblots for their thesis: Dr. Deepak Kumar, Dr. Ruchi Jain, Dr. Sunita Kandhari and Dr. Jashobanta Mahapatra; and postgraduate diploma students who worked on Rorschach inkblots for their projects: Mrs. Nidhi Lathey and Mrs. Varsha Sharma.

I am thankful to my friends Dr. Ajai Kumar Srivastava, Prof. Amool Ranjan Singh, Dr. Kaptan Singh Sengar, Dr. Jaiprakash and Dr. Rakesh Pandey for their contribution in this work.

I am grateful to the psychiatrists who sought for Rorschach based diagnostic formulation for their patients. I am also indebted to thousands of the psychiatric patients who presented themselves for Rorschach technique and enriched my understanding of the inkblots.

Every Rorschachian is indebted to the master Herman Rorschach, Verlag Hans Huber, S.J. Beck, Bruno Klopfer, John E. Exner, Z.A. Piatrowski and those who have contributed a scoring system for the Rorschach. I have a reverence to Indian psychologists particularly Prof. H.S.Asthana, Prof. T.R.Shukla, Dr. N.L.Dosajh, Dr. Pramod Kumar and Prof. B.L. Dubey who worked extensively on inkblots.

I am thankful to Prasad Psycho Corporation, New Delhi for accepting the publication of this manual and taking pain in getting an excellent typeset and graphics to enhance the learning experience.

A subject or patient in this guide is referred by 'he' just for the convenience of presentation. If a subject or patient is of female gender, it could be read as 'she'.

The thoughtful comments and suggestions of wise readers would help me to revise the next edition of this manual. These may be forwarded at Email: mindpowerlab@gmail.com.

Rakesh Kumar
Agra-The City of Taj

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CHAPTER-1: INTRODUCTION

Rorschach Inkblot Test is one of the most important and frequently used test in clinical set ups. This test permits a clinician to formulate psychological diagnosis, understand the personality and provides clues that are useful in psychotherapies. There are a number of quantitative and qualitative indicators which suggest the existence of psychopathology in a patient.

Hermann Rorschach experimented with a series of inkblots and finally came up with the existing set of 10 inkblots. The Rorschach plates are prepared by throwing inks on white sheet and folding it in the middle. In five plates only black ink is used and in remaining five plates chromatic colors have been added. The test is based on the theory of projection which can be illustrated through following example:

Each of us have seen clouds in the sky. For a moment; just visualize a piece of cloud. The shape and size of the cloud is accidental. No body has deliberately cut it in that size and shape. If you were asked to report the objects to which this piece of cloud resembles you would be able to perceive one or the other objects in it. It could be animals, human beings, plants or any conceivable objects. While associating to the cloud, it is quite possible that you might select the entire piece of cloud or part of it and perceive the objects. So it can be derived that if a person is provided with unstructured, accidental forms he would be able to perceive some objects in such stimuli.

If the same piece of cloud is shown to many persons, each of them would be able to associate one or the other objects with the cloud. Now the million dollar question is, if it is shown to more than one person, whether the perceived objects would be identical or there would be differences in the perceived objects. The obvious answer is that there would be differences in the nature and quantum of the perceived objects. So it can be derived that because of individual differences people may perceive different objects in accidental forms.

At last if we were to contemplate, why there would be individual differences in the perceived objects, is it because of the piece of cloud or individual's mind. You would quickly respond correctly, the individual's mind. So the logical deduction is that, it is the mind of the individual which produces different set of associations to the accidental forms. This illustrates the point that the accidental forms

can be used to study the mental functions of an individual. When provided with unstructured stimuli an individual tends to reveal his mental status through the nature of associations.

In every psychiatric disorder, mental functions get disturbed. It may be in the form of emotional disturbances, cognitive changes, behavioral disturbances and so on. Every disorder like, schizophrenia, mania, depression, obsessive compulsive disorder has a characteristic set of symptoms which are essentially deviations from the normal mental functions. Rorschach Inkblot Test is the standardized set of accidental forms which permits detection of changes in mental functions.

The Concept of Inkblots:

You might remember that many children spill ink on a paper and fold it in the middle, sometimes they use a thread, dip it in the inkpot and place on a white sheet, fold the sheet and remove the thread, and open the paper. The result is a blot of ink. An inkblot is the printed portion on a sheet of paper. Every portion of ink is the blot area and all white space irrespective of its position on the paper is the white background area. An inkblot (Non-Rorschach prepared by throwing ink on a white sheet and folding in the middle) is illustrated in the following figure:



Having made the inkblot, children try to perceive some objects in the inkblot and also get associations from their friends. Children do come up with meaningful associations to the random inkblots. This popular observation confirms the assumption that unstructured and accidental forms elicit meaningful associations to the entire blot or the part of it. At this point, you may pause for a moment, inspect the above inkblot and enlist the objects that resemble to this inkblot. You would certainly come up with one or more associations to this inkblot and realize that inkblots do elicit meaningful associations.

Inkblots were the subjects of scientific experimentation even before Rorschach conceptualized them for the study of psychiatric disorders. Bine and Henri in 1895 suggested that inkblots could be used to study imagination and personality traits. Hermann Rorschach began his systematic studies on inkblots in 1911 and got his inkblots published in 1921. Rorschach left for heavenly abode in 1922, his students shouldered the responsibility and spread the work of Rorschach.

Many scoring systems have been developed for Rorschach Test. Following are the most popular systems of scoring in the world: Exner’s comprehensive scoring system, Klopfer & Kelly, and Beck. This manual is primarily based on Beck’s system. The interested readers may like to read the original volumes of Beck to grasp the system in its entirety.

Achromatic and Chromatic Inkblots:

Inkblots can be prepared with black ink only or with hues like red, green blue. The black inkblots are called achromatic and the inkblots having colors other than black are called chromatic. Following inkblots exemplify the concept of achromatic and chromatic inkblots:

.....

Achromatic Inkblots (Non-Rorschach)



.....

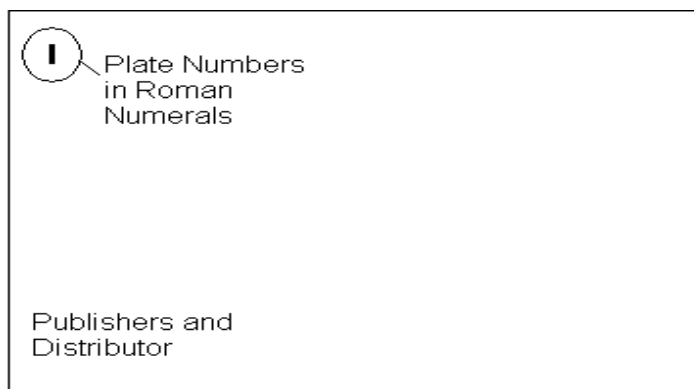
Chromatic Inkblots (Non-Rorschach)



.....

Introduction to Rorschach Plates:

Rorschach test consists a series of ten Plates of 24x17cm size with inkblots printed on them. In the series of ten plates five are achromatic, two plates are black and red; and the remaining three plates are 'Polychromatic' in combination of green, blue, orange and pink blots. The card no, I, IV, V, VI & VII (five cards) are printed in black which are called as Black and White Cards or Achromatic Cards. The remaining cards: II, III, VIII, IX, & X (five cards) contain two or more colors. These are called colored cards or chromatic cards. (Remember the analogy of Black & White TV and Color TV). If you look at the back of the plates you would find two information: at the left top, plate numbers are given in Roman numerals. At the bottom of the left, you would find name of publisher and distributor printed. When a card is held front side facing, the Roman Numeral indicates top position. This is illustrated in following figure:



The scheme of front side of the Rorschach is illustrated below. The arrangement begins with plate no. I and ends at plate no. X in sequential order. You may familiarize yourself with the plates by arranging and re-arranging the front side of the cards in sequence.

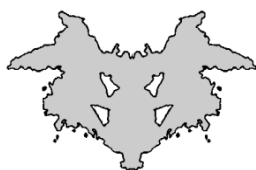


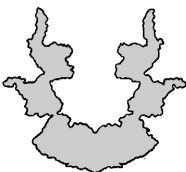
Plate-I: Black (Achromatic)



Plate-VI: Black (Achromatic)



**Plate-II: Black and Red
(Chromatic)**



**Plate-VII: Black
(Achromatic)**



**Plate-III: Black and Red
(Chromatic)**



**Plate-VIII: multi-colored
(Chromatic)**



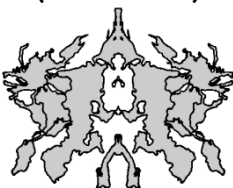
Plate-IV: Black (Achromatic)



**Plate-IX: Multi-colored
(Chromatic)**



Plate-V: Black (Achromatic)



**Plate-X: Multi-colored
(Chromatic)**

CHAPTER-2: ADMINISTRATION

Materials Required for Administration:

1. **Rorschach Plates:** The standard set of 10 Rorschach plates
2. **Location Chart:** A printed single sheet of paper containing 10 Rorschach Images. These are available separately from the test supplier.
3. **Response Sheet:** A specially designed full scape plain sheet of paper with defined columns. You need to prepare it for yourself as per the scheme mentioned below. The meanings of acronyms used are provided at appropriate place in the administration section.
4. **Colored Ink Pen Set:** You could take a set of 6 colored ink pen set may be sketch pen or gel pen. You will need them to mark blot areas on the location chart.
5. **Digital Stop Watch:** You could use your mobile set if it has a stop watch or a stop watch of any make. The wrist watch is not recommended. You will need to record timings during administration of the Rorschach Test.
6. **Rorschach Test Manual:** Any manual including this one and according to the preferred scoring system would do.

Response Sheet

Name of the Subject:

Subject ID:

Date of Administration:

Education:

Age:

C.N*	RT ₁	RT _T	CP	Responses	Enquiry	L	D	C	P

* The symbols used in this response sheet are illustrated at appropriate places in this manual. **CN**= Card No.; **RT₁** = Reaction Time to First Response; **RT_T** Total time spent on a card; **CP** = Card Position (Orientation); **L**= Location (Blot Area); **D**=Determinant (Object Attribute); **C**=Content (Object Category); **P**=Popular (Most Frequent Response)

Rorschach inkblot Test: Location Chart

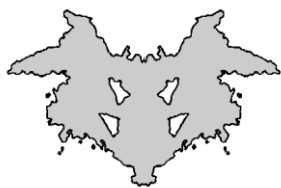


Plate-I

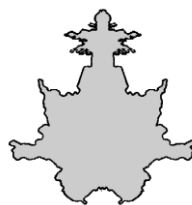


Plate-VI



Plate-II

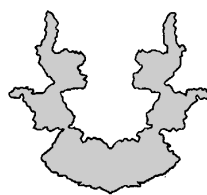


Plate-VII



Plate-III



Plate-VIII



Plate-IV



Plate-IX

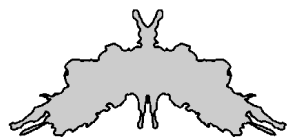


Plate-V

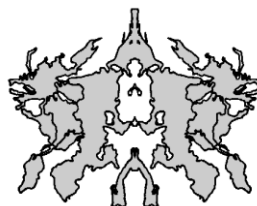


Plate-X

The Concept of Card Positions:

It is the position of the card at which a subject produces a response. It is also called Orientation. The Rorschach plates are presented to a subject in top card positions, however a subject is free to rotate the card and view it in any position. An inkblot serves as the stimulus for a subject to associate. The nature of stimulus changes with each card position. The stimulus in top position is different from the stimulus in bottom or any other position. The retinal image changes with every change in the card position. You may appreciate it by holding a Rorschach plate in various positions and see the change in the stimulus with the change in card position. A Rorschach examiner is required to record all card positions from the subject's relative position. There are four options to record card positions.

1. Top (↑)
2. Bottom (↓)
3. Left (←)
4. Right (→)

**Example of Top Card
Position (↑)**



**Example of Right Card
Position (→)**



**Example of Bottom Card
Position (↓)**



**Example of Left Card
Position (←)**



Exercises in Card Position: Detect the card positions in following inkblots



Plate-I



Plate-II



Plate-III



Plate-IV



Plate-V



Plate-VI

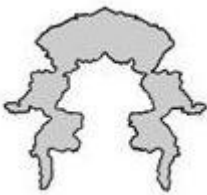


Plate-VII



Plate-VIII



Plate-IX

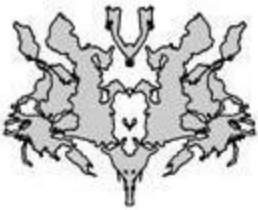


Plate-X

Key	I	II	III	IV	V	VI	VII	VIII	IX	X
	←	↑	↑	→	→	↑	↓	←	↑	↓

Preliminary Preparation:

1. Establish rapport with the subject
2. Collect adequate history
3. Conduct an interview to record the mental status of the subject
4. Handle worries and queries of the subject regarding test and any aspect of the situation

Seating Arrangements:

1. You need to have at least one table and two chairs for the assessment.
2. Get the subject sit on your right hand side chair
3. Do not allow any observer, third person or any family member in the room during test administration

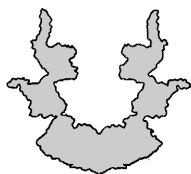
Test Administration

1. It is recommended that you should allow yourself with sufficient space to learn the test. Feel free to explore your own ways. Do not get panicky at any stage for committing errors. Remain comfortable in your learning. For about 10 administrations you should remain in a learning mode.
2. After establishing the rapport, hand over Card No. I to the subject and ask *"Tell me what you see in it"* No detailed instructions are required. **Caution:** never give a clue to the subject that he could see animals/birds/plants and so on. Also do not tell the subject that he could change the card position to produce responses.
3. As soon as you hand over the card to the subject mark Card Number on Response Sheet. Remember that Card Numbers are marked in Roman Numerals I, II, III,...-X.
4. Keep a track of the card positions. Record all the Card Positions of a plate in which the subject holds the card for a few seconds even if no response is produced. When you record all the card positions, the last position is the one in which patient produced a scorable response or returned the card without producing any response. The possible positions are Top, Right, Left, Bottom. If no card position is mentioned, it always means Top position. *Card positions are recorded for every response.* For example:
 - $\uparrow \leftarrow \uparrow \rightarrow$: This sequence of record means that the subject held the card in following position: top, left, top and right. He produced the response at the right position

- ↑: This record of card position means that the subject produced the response in top position without preceding rotations on the card
5. RT₁ (Reaction time to first response) means the time elapsed between presentation of a card to the subject and production of first scorable response. To record it, start the stop watch when a card is presented to the subject and see how much time has elapsed when the subject produced the first scorable response. Let the stop watch continue; do not stop at this point. (Note: if recording of RT interferes in the learning process or makes the subject conscious, you may skip recording the time. It would not significantly affect the overall personality mapping through Rorschach).
 6. When a subject produces a scorable response, record the Response Number in Arabic Numerals – 1, 2, 3 etc in the response column and record the response verbatim.
 7. RT_T (Response Time) is the time elapsed between presentation of the card to the subject and return of the card by the subject. To record this stop the stop watch upon return of the card and record how much time a subject kept the card with him.
 8. Leave a space of about 1 cm between recordings of two cards.
 9. Upon presentation of the next card, reset the response number to 1. That is, on each card the response number would begin from 1.
 10. Continue to present the card and record responses till last response on Card No. X.
 11. This phase of obtaining responses to Card-I to Card-X is called Response Phase or Free Association Phase.
 12. A subject may try to reject a card immediately upon presentation. Tell him “take your time, may be you will see something in it”. In any case, wait for at least two minutes before accepting card rejection.
 13. A subject may become anxious if it is a test of IQ and the like. If the subject expresses such thoughts, tell him that this test helps to understand his nature, it is not a test of IQ.
 14. At times, a subject starts describing some events which are not related to the test, tell him that after finishing this task, he could discuss anything in detail.

15. A subject may have previous exposure to the test and report as such, tell him that repeat tests yield additional and useful information.
16. There is an opinion regarding Rorschach test that it should not be repeated on a subject before six months. In legal and unresolved cases, the test could be repeated even after an interval of a day.
17. A subject may explicitly ask whether the reported percept is right or wrong, tell him that there is no right or wrong answer in the test. Every one could see many different things.
18. A subject may enquire whether he could turn or rotate the card, tell him yes, as you wish. However, never initiate or encourage card turning from your side.
19. Do not interrupt administration by enquiring anything related to response or blot in the response phase. You will have sufficient time to do so during enquiry phase.
20. A subject may keep on producing too many responses to a card. You may obtain 10 or more responses to a card. A lengthy protocol may not enrich an understanding about the subject's psychological functioning. Also it may become cumbersome to score and interpret very lengthy response records. A standard of taking a card back after five responses may be followed. The taking back should be non-threatening. Just tilt a little and make a gentle gesture indicating that you want the card back should be enough to hint the subject to return the card gracefully.
21. Your competence and efficiency will increase with every administration and scoring of Rorschach Test.

Examples of Response Recording:



-
- (1) Two faces looking at each other
 - (2) Thumb



- (1) Two monkeys climbing on a mountain
- (2) Butterfly

Enquiry:

After completing the response phase you would be required to conduct an enquiry of each response produced by the subject. There are two primary quest of an examiner in the enquiry phase

- I. to delineate the blot area used by the subject to produce a response
- II. to obtain a description of the attributes of the perceived object.

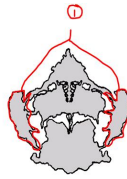
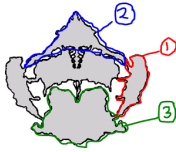
To accomplish the above tasks of the enquiry, an examiner primarily asks two questions from the subject:

- I. **Where** on the inkblot, the perceived object was seen?
- II. **Why** the blot area looks like the perceived object?

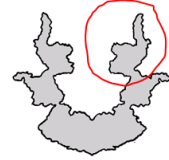
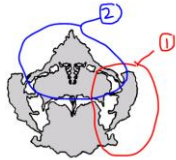
Guidelines for Enquiry: The enquiry proceeds in following manner:

1. Remain gentle through out the enquiry phase.
2. Conduct enquiry response to response. That is, complete the enquiry for a response and then move to the next response.
3. The enquiry phase begins immediately after completion of the response phase. However, a subject may be allowed to relax for about five minutes. The enquiry should not be postponed for hours/days
4. The enquiry could be conducted backward, that is, from Card-X to Card-I. There is flexibility in this rule. One could proceed forward also from Card-I to Card-X.
5. For backward enquiry, hand over card-X to the subject, repeat response no. 1 to the subject and ask where you saw it. Get a clear tracing to the blot area used. Some subjects keep on pointing major areas by finger for which examiner may not be sure about the blot area used. This can be overcome by giving a tracing object like blunt pencil or top side of the pencil and ask the subject to trace the blot area with this object.
6. When a subject traces the blot area and you are sure of it, then immediately mark the traced area on the Location Chart and assign it the same response number
7. For neat and clean tracing on location chart, you could use colored gel ink pen. You may fix the color for each response number. That is, red ink for response number 1, blue ink for response number 2, green ink for response number 3 and so on. On every card you may use this pattern for uniformity. Remember, this is just optional guideline. You may use any ink for any response. However, black ink should never be used. The location charts are printed black and if you use black ink for tracing the location chart, the tracing may not be visible. While tracing on the location chart, trace along the contours.

Examples of Recommended Blot Area Tracing:



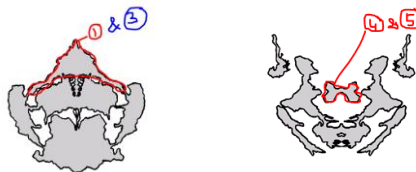
Examples of Not-Recommended Blot Area Tracing (Beyond Contours)



-
8. The Rorschach images are symmetrical. Some subjects produce responses in pairs incorporating both sides of the blot. In such cases you may trace both sides of the blot or trace any lateral side.
 9. At times, the subjects produce compound responses. These are the responses having more than one object in a single response. For example, two bears climbing on a mountain (bears + mountain). Two ladies filling water in an earthen pot (Ladies + Water + Earthen Pot). In such instances you could trace all the objects and mention the name of each object on the location chart itself.
 10. Many psychotic patients or depressed patients may not be able to provide specific details of the blot area; they may totally or partially fail to trace the blot area. No problem, this would be handled in qualitative analysis of Rorschach.
 11. Some patients may deny having seen the perceived object on the blot area. Enquiry can not be conducted for such responses. Just write Response Rejection for these denied responses and move forward.
 12. While enquiring about blot area, the card should be given in the same position in which the response was produced by the subject. That is, if the response was produced in bottom card position then the card should be rotated to the bottom position to enquire about the blot area.
 13. In some patients, you would find that they either replaced the response given in response phase or produce additional new responses in the enquiry phase. To deal with such a situation, you should record these replaced/new responses

in the enquiry phase for the space available in enquiry column for the card. Remember, no enquiry is conducted for these replaced/new responses. Just mention them and direct your enquiry to remaining responses of the response phase.

14. A subject may produce more than one response to the same blot area. In such cases, mention the respective response numbers on the traced blot area. Following figure illustrates such instances:



15. After tracing the blot area, examiner moves to the second quest to ascertain the attributes of the perceived object.
16. Take out the response sheet to record the enquiry details.
17. Now ask the subject **“in what way”** it looks like that. Whatever the subject says record verbatim in the enquiry column of the response sheet. Some subjects become irritable when an examiner frequently asks **“Why”** it looks like that. Frequent use of **“Why”** is not recommended.
18. Manage the row space carefully to record the enquiry details in the column for each response.
19. Never be directive in the enquiry or give any clues to the subject. For instance, do not ask whether it looks like because of color, shape or there is any movement in it.
20. Remember that attribute enquiry is being conducted for the object perceived by the subject not for the blot area or the object as such.
21. If insufficient details are provided by a subject in the enquiry of attributes, then you may say “tell me more about it”, “what else makes it look like that”. If still you find any problem in getting adequate details, in the last you may say to the subject **“Show me how it looks like that to you”**, record the verbatim and move to next response.

22. Examples of Correct Attribute Enquiry Questions:

- a. Response: Two monkeys
- b. Attribute Enquiry: Examiner: In what way it looks like two monkeys? Subject: looks like that, here are their hands, feet, face. Examiner: Tell me more about it. Subject: holding a tree.
- a. Response: Two human being
- b. Attribute Enquiry: : Examiner: In what way it looks like two human beings Subject: you see here are their faces, trunk, arms and legs. Examiner: Tell me more about them. Subject: nothing. Examiner: Show me how it looks like that to you. Subject: you see here are their faces, trunk, arms and legs. They are bent forward as if lifting something.
1. Response: Flowers
2. Attribute Enquiry: Examiner: In what way it looks like flowers. Subject: The shape resembles to flowers. Examiner: Tell me more about them. Subject: Here are petals and looks similar to flowers. Examiner: Show me how it looks like that to you. Subject: You see here are petals, just look like pink flowers

23. Examples of Incorrect Attribute Enquiry Questions:

- c. Response: Two monkeys
 - d. Attribute Enquiry: Examiner: Tell me if they look like monkeys because of shape, are they doing something.
 - c. Response: Two human being
 - d. Attribute Enquiry: : Examiner: Tell me if they are doing or feeling something.
 3. Response: Flowers
 4. Attribute Enquiry: Examiner: Tell me if they look flowers because of colors or shape. Are they pink, yellow or red?
24. The test administration ends with the enquiry of last response.

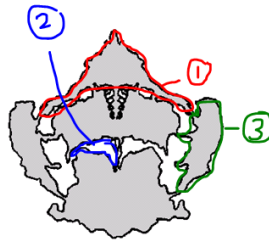
CHAPTER-3: SCORING METHOD

1. To score a Rorschach protocol you would need Rorschach Plates, response sheet, location chart and test manual. Most of the scoring can be done through this manual. If you wish to follow other methods like Beck, Exner, Klopfer & Kelly; then you would need their manual.
2. The scoring is done response to response in a row. That is, complete the scoring of response no. 1 and then move to next response.
3. The scoring of the protocol begins with response no.1 on the first card and proceeds to other responses and cards until the completion of last response to card no. X.
4. Two types of scoring is done for every Rorschach protocol – Quantitative and Qualitative.
5. For quantitative scoring there should be at least 15 scorable responses in a protocol. In any case there should not be less than 10 responses to score the protocol quantitatively.
6. For a protocol having less than 10 responses, only qualitative analysis is done. The summary for quantitative variables may be skipped.

Quantitative Scoring: Quantitative scoring is done for following variables:

- a. Blot Area
- b. Object Attributes:
 - i. Shape
 - ii. Human Action
 - iii. Chromatic Color
 - iv. Achromatic Color
 - v. Tactile Response
 - vi. 3 Dimensional Response
- c. Object Category
- d. Most Frequent Response

The Concept of Blot Area: It is the area of an inkblot extracted by a subject to produce a response. This area corresponds to the tracings done by you on the location chart for the responses produced by the subject. The examples of blot area are given in following tracing:

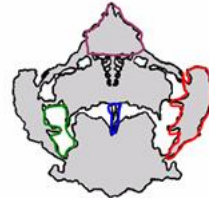


In this inkblot, tracing is done for three responses labeled by their respective numbers from 1-3. This depiction indicates that three scorable responses were produced on this inkblot on three separate locations.

There are two possibilities for extracting a blot area to produce a response. (i) either the subject will select the entire blot (ii) or part of the blot. If entire blot area is selected then we call it as Total Blot Area. If only part is selected then we call it as Part Blot Area. This is illustrated in following figure:



Total Blot Area



Part Blot Area

Break up of Part Blot Area:

1. Part Blot Area can further be broken into two types. (a) Common Blot Area, and (b) Uncommon Blot Area.
2. **Common Blot Area:** The Part Blot Area which is commonly selected by the subjects is called Common Blot Area. Such areas are easy to perceive and attract immediate attention of the subjects. **(Caution:** It is a mistake to consider larger Part Blot Area as the Common Blot Area. Size is not the decisive factor for Common Blot Area. It is the frequency with which it is selected for responding on the inkblots. It may also be called as *Frequently Seen Part Blot Area*). The examples of

Common Blot Area are given in following inkblots. Common Blot Area is indicated by red tracing. Notice that even a tiny area also constitutes the Common Blot Area. Refer to corresponding Rorschach Test Plate while examining the blot areas.



3. **Uncommon Blot Area:** The Part Blot Area which is uncommonly selected by the subjects is called Uncommon Blot Area. Such areas are not easy to perceive and attract less attention of the subjects. (**Caution:** It is a mistake to consider Smaller Part Blot Area as the Uncommon Blot Area. Size is not the decisive factor for Uncommon Blot Area. It is the frequency with which it is selected for responding on the inkblots. It may also be called as *Infrequently Seen Part Blot Area*). The examples of Uncommon Blot Areas are given in following inkblots. Uncommon Blot Areas are indicated by red tracing. Notice that even a larger area also constitutes the Uncommon Blot Area.



Exercises in Part Blot Area: Identify the part blot areas in following inkblots. You have two options to name (a) Common Blot Area (b) Uncommon Blot Area. While identifying the part blot areas, refer to traced part in the Rorschach Plates. Remember that Common Blot Areas are easy to perceive and attracts immediate attention of the subjects.

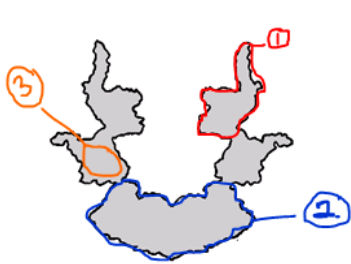


Plate-VII

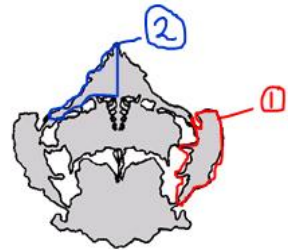


Plate-VIII

Key	Plate-VII	Plate-VIII
Common Blot Area	Response No. 1 & 2	Response No. 1
Uncommon Blot Area	Response No. 3	Response No. 2

Guidelines to Resolve Part Blot Area Issues:

1. The Part Blot Areas are determined statistically.
2. Whether a particular part blot area is a Common Blot Area is not an arbitrary decision. A Rorschach examiner does not have liberty to label any area as Common Blot Area.
3. Complete list of Common Blot Areas and Uncommon Blot Areas is available in Beck et al. (1961). A list of Blot Areas is also presented in this manual.

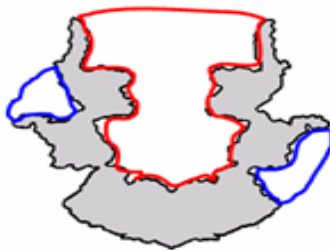
4. Remember that if any part blot area is not listed in Beck's manual then it is designated as Uncommon Blot Area. It should not be scored as Common Blot Area even if it appears to you to be easily perceptible.

Scoring Symbols for Blot Areas:

1. While preparing summary of the Rorschach scoring you will be required to use symbols. Every quantitative scoring has its own symbol.
2. Total Blot Area is also known as Whole and its symbol is W.
3. Common Blot Area is also known as Obvious Detail, Major Detail. The Symbol for Common Blot Area is D.
4. The Uncommon Blot Area is also known as Minor Detail, Rare Detail. The Symbol for Uncommon Blot Area is Dd.

The Concept of White Background Area:

1. Recall that inkblots are prepared by throwing ink on white paper. All ink portions constitute the inkblot. This inkblot is the figure against white background of the paper. This white background may lie beyond the contours of inkblot and also inside the inkblot.
2. All white background irrespective of its location outside or inside the inkblot, is called White Background Area.
3. **Examples of White Background Area:** The tracings in following figure represent the White Background Area.



White Background Area

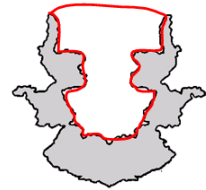
4. While responding on the Rorschach plates a subject may respond exclusively to inkblot, White Background Area or combine the inkblot and white background area.
5. The extractions of these three possibilities are depicted in following figures:
 - a. Only Inkblot
 - b. Inkblot + White Background Area
 - c. Only White Background Area



Only Inkblot



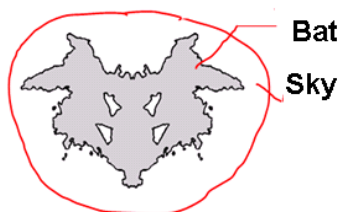
Inkblot + White
background Area



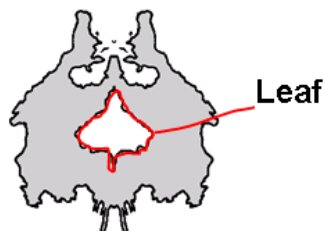
Only White
Background Area

6. Whatever is the nature of White Background Area, it remains the same for the purposes of scoring.
7. There are some 'only white background areas' that are commonly extracted by the subjects. Also there are a few 'only white background areas' that are uncommonly extracted by the subjects for responding.
8. The symbol for White Background Area is S, derived from the white space.
9. Remember that White Background Area (S) is never scored as standalone. It is an addition and hence suffixed to Total Blot Area, Common Blot Area or Uncommon Blot Area, whatever is the case.
10. The possible scoring for white background area is as under:
 - a. Total Blot Area + White Background Area = Ws
 - b. Common Blot Area + White Background Area (or only Common White Background Area) = Ds
 - c. Uncommon Blot Area + White Background Area (or only uncommon white background area) = Dds.

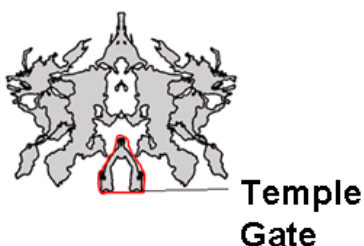
11. It is not recommended to prefix the white background area symbol to the Blot Area Symbols e.g. sW, sD, sDd.
12. The examples of White Background Area scoring is provided in following figures:



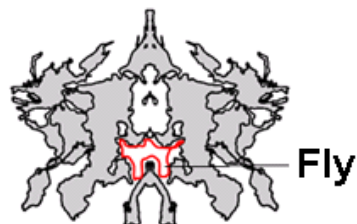
Total Blot Area + White Background Area = Ws



Only White Background Area = Ds



Common Blot Area + White Background Area = Ds



Only Uncommon White Background Area = Dds

13. All possible scoring for Blot Areas, thus would be as under:
 - a. Total Blot Area = (W)
 - b. Common Blot Area = (D)
 - c. Uncommon Blot Area = (Dd)
 - d. Total Blot Area + White Background Area = (Ws)
 - e. Common Blot Area + White Background Area (or Select White Background Areas only) = Ds
 - f. Uncommon Blot Area + White Background Area (or select White Background Areas only) = Dds
14. A response will carry only one scoring for Blot Area. This scoring is entered in the response sheet in the column designated as L (Location). For example, a subject responded as Butterfly to Total Blot Area of Plate-I, then enter W in the L column in the same row in which response is entered in the response sheet.
15. Cut off W is never scored in this system.

The Concept of Perceived Object Attributes:

1. A Rorschach examiner has to be adept in deciphering attributes of a perceived object.
2. An object attribute is the characteristics of the perceived object which is revealed by the language of the subject in response and enquiry combined together. Remember, an attribute has to be perceived for the object on the inkblots for accurate scoring. If an attribute is not perceived on the inkblots then that attribute is not scored. For example, bats are black by nature. If a subject's language does not indicate that this blackness is being perceived on the blot, the attribute of blackness is not scored.
3. Response attribute is otherwise known as determinant.
4. At times more than one object attribute may be present in a single response. Every object attribute present should be scored.

Explanations of Various Object Attributes:

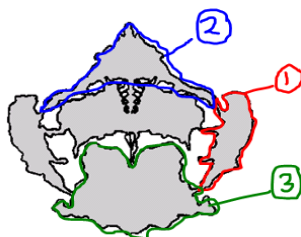
Object Attribute – Shape: This object attribute is encountered most frequently. When a subject qualifies a perceived object by its parts, size, structure and shape, then this attribute is scored. Also if no other object attribute is evident, then the attribute of shape is scored. The symbol for shape is F derived from Form. The examples are provided below:



Response-1: Human Face

Enquiry: Looks like a face, here is the forehead, eyes, mouth, chin

Comment: Only parts of the face have been mentioned. Hence the attribute is Shape.



Response-1: Mice

Enquiry: Looks like a mice. Legs, face, tail

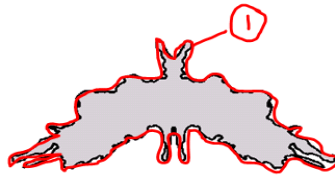
Comment: Only parts of mice have been mentioned. Hence, the attribute is Shape.

The Concept of Shape Appropriateness:

1. Shape appropriateness is determined for each and every response attribute.
2. In shape appropriateness, it is judged if shape of the perceived object resembles the contours of the extracted blot area.

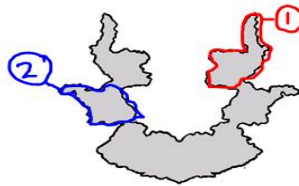
3. If a perceived object match with the contours of the extracted blot area then it is to be scored as Shape Appropriate.
4. If a perceived object does not matches with the shape of the blot area, it is scored as Shape Inappropriate.
5. The scoring for Shape Appropriate response is the + sign
6. The scoring for shape inappropriate responses is the – sign.

Examples of Shape Appropriate Objects:



Response-1: Bat

Comment: The perceived object-Bat matches/resembles with the shape of the Total Blot Area selected for this response. Hence, the scoring will be Shape Appropriate.



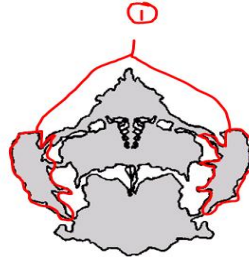
Response-1: Human Face

Comment: The extracted part blot area for response-1 resembles the shape of human face, hence the scoring will be Shape Appropriate.

Response-2: Thumb

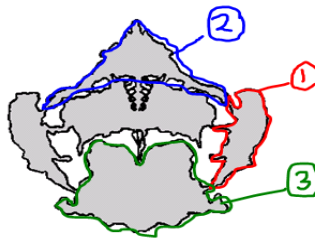
Comment: The extracted part blot area for response-2 matches with the shape of thumb, hence it shall be scored as Shape Appropriate.

Examples of Shape Inappropriate Objects:



Response-1: Two Ladies

Comment: The extracted blot area does not match with human figures; hence it shall be scored as Shape Inappropriate Response.



Response-3: Monkey

Comment: The part blot area extracted for Response-3 (monkey) does not match with the shape of a monkey, hence it shall be scored as Shape Inappropriate Response.

7. If a perceived object is shape appropriate then just add a plus sign after the scoring of response attribute
8. If a perceived object is shape inappropriate then just add a minus sign after the scoring of response attribute.
9. If the attribute is shape and the perceived object is shape appropriate then score it as F+ and enter it in the D column of response sheet.
10. If the attribute is shape and the perceived object is shape inappropriate then score it as F- and enter it in the D column of response sheet.

11. Guidelines to Determine Shape Appropriateness:

- a. You yourself can judge the appropriateness of a perceived object.
- b. If you are in doubt about the shape appropriateness, then show the extracted blot area to at least three of your friends and ask them if the perceived object resembles the shape of the blot area. Score the response based upon the opinion of majority. If majority says, appropriate score it as plus, otherwise score it as minus.
- c. In case of extreme uncertainty you may skip the scoring of shape appropriateness.
- d. A comprehensive list of shape appropriate and shape inappropriate responses for each blot areas on every Rorschach Plate is provided by Beck et al. (1961).
- e. A sample list of shape appropriate and inappropriate responses is also provided in this manual.

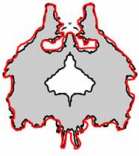
Object Attribute – Human Action:

1. A human being, mythological being, animal or inanimate object engaged in any human muscular action, emotion, posture is scored as Human Action attribute. The symbol for Human Action is Capital (upper case) M derived from Movement.
2. Human Action is scored if any of the following conditions is met:
 - A. The perceived object is a human being and engaged in any action, posture, emotion. For example, a human is sitting, lying, bent forward, smiling, facing other human being, talking, feeling sad, laughing, hiding behind a tree.
 - B. If the perceived object is an animal who is engaged in the activities of human being, then Human Action is scored. For example, an animal laughing, sad, in competition with other animal, dancing, thinking.
 - C. If an animal is engaged in an activity which is not typical of its species, then Human Action is scored. For example, an ant is barking, a dog is climbing the tree, a rat is flying
 - D. If the perceived object is an inanimate object which is made to move by a human being either directly or

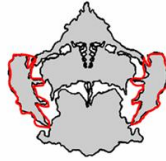
through any device. A flying airplane, a toy dancing through the remote

- E. All human emotions like sadness, depression, joy, disgust, anxiety, fear are scored as human action. Such emotions may constitute standalone response or may be attributed to a human being or an animal.
 - F. Human action is also scored for super-human beings or mythological entities like a fairy is dancing, Spiderman climbing a wall, flying baital.
 - G. The actions in photographs, statues, dolls, comics are also scored as human action. For example, a statue in saluting posture.
 - H. An animal performing the trained activities of a human being would be scored as M. For example, dancing bears.
 - I. Human action may be present in complete human being or part of the human being. For example, grinding teeth, staring eyes, fist, finger pointing at something.
 - J. Human action may be present in any blot area i.e total blot area, part blot area or white background area.
3. Human Action is not scored in following conditions:
- A. If the perceived object is a human being and not engaged in any action, posture, emotion; then Human Action attribute is not scored. For example, a human being, here are his legs, arms, face. (**Comment:** There is no reference to any action, hence human action attribute is not scored).
 - B. If the perceived object is an animal engaged in an activity typical of its species anatomically, human action is not scored. For example, a barking dog, a monkey climbing on a tree, a running rabbit, bird flying, a sleeping cat. (**Comment:** All these are species specific behaviors hence human action is not scored).
 - C. If the perceived object is an inanimate one and engaged in its own actions not mediated by human being, then Human Action is not scored. For example, wind is blowing, thunder storm, leaves moved by wind, flowing water
4. **FM** and lower case **m** are never scored in this system.

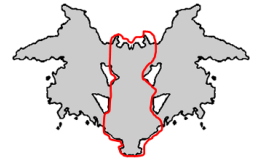
Examples of Human Action Responses:



Two Persons
Dancing



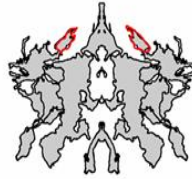
Two Rats in
Competition



A Lady, Raised
hands



Two Ladies
Looking



Angry Birds



Two Dancing
Dolls

Examples of Responses Not Scored as Human Action:



Two Rats
Climbing a Tree



A Flying Bat



Human Faces

Object Attribute – Chromatic Color:

1. The attribute of Chromatic Color is scored when the perceived object is qualified with any color (hue) other than the black and white spectrum. Remember the analogy of Color TV and, Black & White TV. Red, Green, Yellow, Blue, Violate, Purple etc. are included in chromatic colors.
2. The symbol for chromatic color is C derived from Color.
3. **Examples:** Red blood, green leaf, yellow flower, pink shirt, red blood spots, blue ocean are scored as chromatic colors.
4. Black cloud, white shirt, gray clothe are not scored as chromatic color.
5. A response may comprise of a shapeless percept having an attribute of chromatic color or there may be an admixture of

shape with chromatic colors. So the scoring of chromatic color could take three forms: **C**, **CF**, **FC** depending on the extent to which shape contributes to the perceived object.

6. The language of the subject in response and enquiry is the guide for scoring C, CF or FC.
7. When a subject says that the perceived object looks like that only because of chromatic color and shape has no contribution in the percept, then pure C is scored. For example, this is blood because it is red; fire because it is yellowish. Pure C responses are infrequently observed in a protocol.
8. When there is an admixture of shape, it is to be determined what is primary in the two: chromatic color or shape. If chromatic color is primary then the scoring is CF. If the shape is primary and chromatic color is secondary then the scoring would be FC. For example, red spot, because it is red (CF), red rose because its shape is of flower and red color is filled in it (FC), spread colors of Holi (CF).
9. To clarify the contribution of shape in the percept, ask the subject if color of this extracted blot area is changed, then the percept would remain the same or it would change. For example, on Card-X lower green part, the perceived object is a green leaf. Ask the subject if it is filled with red color, still it would be a leaf or something else. If the subject says, it would not be a leaf, then the scoring shall be CF; if he says that it shall still be a leaf then the scoring would be FC.
10. In case of doubt, differentiate the nature of the perceived object. The perceived object could belong to either of following two categories: **(a) Accidental Shape (b) Defined Shape**. The accidental shape percepts are the percepts which do not have any specific shape; their contours can take any shape, the emerging shape is purely accidental. For example, a blood spot can take any shape; it could appear round scattered and the like. If you ask 10 persons to draw the contours of red blood spot, it is likely that every one would come up with different drawings. Their drawings are unlikely to match. The defined shape percepts on the other hand have well defined contours. For example, a heart; if you ask 10 persons to draw a heart, it is likely that the drawings of most of the persons would match.
11. For accidental shape percepts having chromatic color attribute, you may score as CF and for defined shape percepts

having chromatic color attribute, you may score as FC. For example, red lungs (FC), yellow flame (CF).

12. Pure C responses are infrequent. If there is a confusion whether a percept is a pure C or CF, you can always safely score it as CF.
13. If still there is any confusion between CF and FC in a response, look into the total protocol. If a person has more FC responses then score such a response as FC; if a person has more CF responses in a protocol then score such a response as CF.

Examples of Chromatic Colors:



Red Bowtie
(FC)



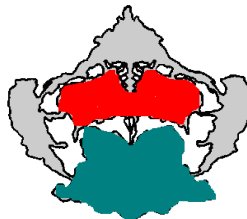
Modren Art
filled with
Colors (CF)



Red Blood I

14. It must be differentiated that if the chromatic color is the attribute of the perceived object or just a location specifier. A location specifier is used by the subjects to point out the blot area. For example, this red portion is a cap; this green one is an insect, this yellow one is a lion. In these examples, chromatic color is used to point out the extracted blot area for responding. These are not the attribute of the perceived object. Location spcifiers are not scored chromatic colors.

Examples of Locations Specifiers:



- a. This red part is a flag
- b. This green part is a lion

Object Attribute – Achromatic Color:

1. The attribute of Achromatic Color is scored when the perceived object is qualified with the black and white spectrum. Red, Green, Yellow, Blue, Violate, Purple etc. are not included in achromatic colors.
2. The symbol for achromatic color is upper case Y.
3. **Examples:** Black cloud, gray hill, white water are scored as achromatic colors.
4. Red blood, green leaf, yellow flower, pink shirt, red blood spots, blue ocean are not scored as achromatic colors.
5. It must be differentiated that if achromatic color is the attribute of the perceived object or just a location specifier. A location specifier is used by the subjects to point out the blot area. For example, this black portion is a cat; this gray portion is an insect, this white portion is a lake. In these examples, achromatic color is used to point out the extracted blot area for responding. These are not the attribute of the perceived object unless otherwise indicated. Location spcifiers are not scored as achromatic colors.
6. There may be an admixture of shape in Achromatic Color responses and it may be primary or secondary. However, you may skip the scoring of shape in achromatic color responses. The recommended scoring is pure Y for all instances of achromatic colors.

Examples of Achromatic Color Scoring:



Black Cloud (Y)

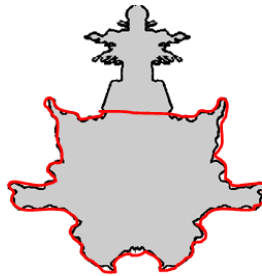


White Rabbit (Y)

Object Attribute – Tactile Response:

1. If a perceived object is described in terms of the sense of touch, then tactile attribute is scored.
2. The symbol for Tactile attribute is Capital (upper case) T derived from Texture.
3. To score tactile attribute, the subject's language should contain the word like rough, hard, soft, warm cool and so on which implies a sense of touch.
4. Any reference to sense of pain, temperature, touch is suffice for scoring as tactile attribute.
5. There may be an admixture of shape in Tactile responses and it may be primary or secondary. However, you may skip the scoring of shape in Tactile responses. The recommended scoring is pure T for all instances of Tactile responses. Examples: rough clothe, hard rock, soft pelt, cool snow, warm water.

Example:



Soft Skin of an Animal (T)

Object Attribute – 3 Dimensions (3D):

1. The object attribute 3D is scored when the perceived object contains the distance, depth, height, insularity or reflection
2. The symbol for 3 Dimension attribute is V derived from Vista.
3. There may be an admixture of shape in 3D responses and it may be primary or secondary. However, you may skip the scoring of shape in 3D responses. The recommended scoring is pure V for all instances of 3D responses.

4. The examples of 3D are: a person standing on a mountain, lake surrounded by hills, reflection of a dog in water, hill top, cave, well.



Mountain Top (V)



Cave (V)

Compound Responses: A response may contain a single object or there may be two or more objects. For example, a bat; two ladies, a butterfly, two rats, contain only single objects. When only a single object is present in a response it is called as Simple Response. When two or more objects are present in a single response it is called as Compound Response. For example, (1) two ladies having a pot in their hands (Ladies + Pot), (2) a rat climbing on a tree (Rat + Tree), (3) a person standing on a mountain (Person + Mountain). In compound responses, subjects usually point out separate blot areas for each objects perceived. For example, a person climbing on a mountain; a subject is likely to point out an area for the person and another area for the mountain. The compound responses may warrant for two or more scoring for the same response. For example, (1) D M+ H (for ladies) and D F+ Ms (for pot) (2) D F+ A (for rat) D F+ Bt (for tree); (3) D M+ H (for person) and D V+ Ls (for mountain).

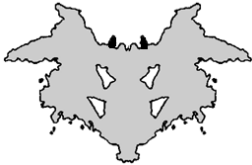
Summary of Scoring Symbols			
Blot Areas (BA)	Object Attributes (OA)	Important Object Categories (OC)	Most Frequent Responses (MF/P)
W	F (F+. F-)	H	P
Ws	M (M+. M-)	Hd	
D	C (CF+. CF-. FC+.	A	
Ds	FC-. C+. C-)	Ad	
Dd	Y (Y+. Y-)	An	
Dds	V (V+. V-)		
DW	T (T+.T-)		
DdW			

CHAPTER -4: BLOT AREAS AND DETERMINATION OF SHAPE APPROPRIATENESS

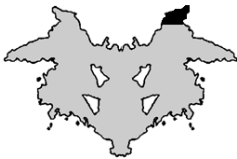
1. Beck et al. (1961) have prepared a comprehensive listing of blot areas and their corresponding shape appropriate and inappropriate responses. Asthana (1976-1977) has prepared Indian norms for shape appropriate and inappropriate responses, which is published by Mishra et al (1996).
2. For illustration purposes, blot areas and corresponding sample shape appropriate and shape inappropriate responses are derived from the work of Kumar (2002), Jain (2002), Mahapatra (2007) and personal experience.
3. To reiterate, the guidelines for determination of shape appropriateness of the perceived objects are as under:
 - a. If the shape of the extracted blot area matches with the shape of perceived object, then it is a shape appropriate response and marked by a plus (+) sign.
 - b. If the shape of the extracted blot area does not match with the shape of the perceived object, then it is a shape inappropriate response and marked by a minus (-) sign.
 - c. Your own judgment regarding shape matching may be suffice in many instances. If you are in doubt, you could seek the opinion of at least three normal persons regarding appropriateness of the shape and go by majority opinion. The procedure is to show the extracted blot area and tell the perceived object to the judges and ask them if the shape of the perceived object matches with the shape of the extracted blot area. If answer is yes in majority award a plus (+) sign else mark it as minus (-).
 - d. The other guideline for judging shape appropriateness is look in the listing of the extracted blot area, if the perceived object is listed in the table, mark the sign as present in the listing. If the perceived object is not listed in the table, look if any object similar in shape is listed. If you find an object similar in shape listed in the table, award the sign present for the similar shape.

- e.** If you find that blot area is not available in the table of the blot areas, then you would not find the listing for shape appropriateness for such areas. In such instances, you would have to rely on your own judgment or the judgment of majority.
- f.** Do not get panicky if you commit any mistake in labeling a blot area and the shape appropriateness. You will continue to learn from every protocol.
- g.** Replace W for TBA (Total Blot Area), D for CBA (Common Blot Area) and Dd for UBA (Uncommon Blot Area) while scoring and summarizing a protocol.
- h.** The most frequent responses have been integrated in parentheses.

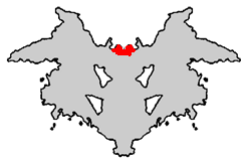
**Plate-I: Common Blot Areas (CBA)
and Uncommon Blot Areas (UBA)**



CBA-1



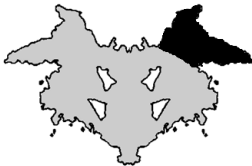
CBA-5



UBA-2



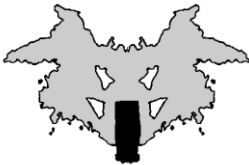
CBA-2



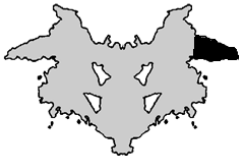
CBA-6



UBA-3



CBA-3



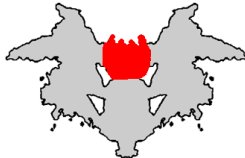
CBA-7



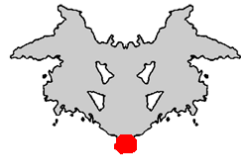
UBAs-4



CBA-4



UBA-1



UBA-5

TBA= W; CBA=D; UBA=Dd

**Plate-I: Sample Shape Appropriate (+)
and Shape Inappropriate (-) Responses**

<u>TBA</u>	<u>CBA-1</u>	<u>CBA-3</u>	<u>CBA-5</u>
- Abdomen	- Ants	+ Bell	+ Bird
+ Airplane	+ Antennae	- Brain Stem	+ Cap
+ Angel	- Apes	- Candle	+ Head- bat,
- Ant	+ Bird Heads	- Face	bird,
+ Bat (P)	- Birds	- Gun	dog, human
- Bear	- Bones	+ Human Form	+ Tortoise
+ Bee	- Bugs	(P)	
+ Beetle	- Clip	- Insect	<u>CBA-6</u>
+ Bird, winged	+ Crabs	+ Legs	+ Beak
+ Bones, Pelvic	+ Dress	- Nose	+ Bird
Griddle	+ Hands	- Penis	+ Dog's face
+ Butterfly (P)	+ Horns	- Snake	- Mountain
- Chest	+ Humans	- Tree	
+ Crow	- Insects	- Vagina	<u>CBA-7</u>
+ Crown (↓)	- Rock	+ Vase	- Arm
+ Door			- Arrow
+ Eagle	<u>CBA-2</u>	<u>CBA-4</u>	+ Beak
+ Face, Animal;	- Airplane	- Animal	+ Flag
Human	+ Angel	- Ant	- Horn
+ Fairy	- Bat	- Bee	- Insect
- Flag	- Beetle	+ Beetle	- Tree
+ Fly	+ Bird	- Bird	+ Wing
- Frog	- Cat	- Bone	
+ Insect	- Chicken	- Cat	<u>UBA-1</u>
+ Kite	- Cow	+ Crab	- Scorpion
- Leaf	+ Dancer	- Face	+ Insect
- Lungs	+ Demon	- Fish	- Face
- Map	- Donkey	- Fly	+ Vessel
+ Moth (P)	+ Elephant	- Frog	
+ Shield	- Fish	+ Human Form	<u>UBA-2</u>
- Spider	+ Human	(P)	+ Breast
- Tree	- Leaf	- Lamp	+ Hills
- Tortoise	+ Map India (P)	+ Monster	+ Shivling
+ X-ray, Chest	- Pig	- Plant	
	+ Soldier	- Reptile+	<u>UBA-3</u>
	- Tree	+ Spider	+ Bell
	- Wolf	- Spine	+ Cockroach
		- Turtle	+ Penis
		+ Violin	

TBA= W; CBA=D; UBA=Dd

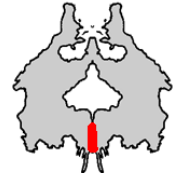
**Plate-II: Common Blot Areas (CBA)
and Uncommon Blot Areas (UBA)**



CBA-1



CBA-5



UBA-2



CBA-2



CBAs-6



UBA-3



CBA-3



CBA-7



UBA-4



CBA-4



UBA-1

TBA= W; CBA=D; UBA=Dd

**Plate-II: Sample Shape Appropriate (+)
and Shape Inappropriate (-) Responses**

<u>TBA</u>	<u>CBA-2</u>	<u>CBA-5</u>	<u>UBA-1</u>
- Bat	+ Bird	+ Animals	- Mouth
+ Bears	+ Blood	- Bat	- Leg
- Bird	+ Boot	- Bird	
+ Butterfly(↓↑)	+ Cap	+ Butterfly	<u>UBA-2</u>
+ Cartoons	- Human	+ Fan	- Human
+ Devils	- Kidney	+ Human, Two	+ Vagina
- Flower		+ Lake	
- Fly	<u>CBA-3</u>	- Lungs	
- Heart	- Ant	+ Pelvis	<u>UBA-3</u>
+ Human, Two (P)	- Bird	- Stomach	+ Blood
- Lungs	+ Blood	- Uterus	+ Fire
- Map	- Bones	<u>CBAs-6</u>	
+ Santa Clause	+ Butterfly (P)	+ Airplane	<u>UBA-4</u>
	- Crab	- Bird	+ Nails
<u>CBA-1</u>	- Fish	+ Bowl	+ Teeth
+ Ape	- Kidney		
+ Bear (P)	+ Moth (P)	+ Building	
- Bird	+ Sun	- Butterfly	
+ Bull	+ Vagina	- Cat	
- Cat	+ Volcano	+ Cave	
- Clown	- Uterus	+ Gate	
+ Cow		- Heart	
+ Demon(↓)	<u>CBA-4</u>	+ Lake	
+ Dog (P)	- Bottle	+ Lamp	
+ Elephant	- Candle	+ Pond	
- Fish	+ Hands	+ Rocket	
+ Hare	- Mountain	- Stomach	
- Leaf	+ Temple	+ Tunnel	
+ Monster (↓)	+ Tower	- Uterus	
- Pig	+ Tree	<u>CBA-7</u>	
+ Rabbit	- Vagina	+ Human From	
- Tiger		(P)	
- Tree		+ Joker	
- Turtle			
+ Wing			

TBA= W; CBA=D; UBA=Dd

**Plate-III: Common Blot Areas (CBA)
and Uncommon Blot Areas (UBA)**



CBA-1



CBA-5



CBA-9



CBA-2



CBA-6



CBA-10



CBA-3



CBA-7



UBA-1



CBA-4



CBA-8



UBA-2

TBA= W; CBA=D; UBA=Dd

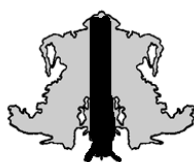
**Plate-III: Sample Shape Appropriate (+)
and Shape Inappropriate (-) Responses**

<u>TBA</u>	<u>CBA-3</u>	<u>CBA-5</u>	<u>CBA-10</u>
- Animal	- Bat	+ Bones	+ Balls
- Ant	- Bird	- Chest	+ Baskets
+ Art, Modern	+ Blood	- Mountain	- Eyes
- Butterfly	+ Bowtie (P)	+ Ribs	- Face
- Face	- Brain	- Vagina	- Hat
- Flower	- Breasts		- Lungs
+ Fly	+ Butterfly (P)	<u>CBA-6</u>	+ Stones
+ Frog	- Crab	+ Airplane	+ Trees, Two
- Insect	+ Dumroo	- Bird	(P)
+ Mountain and	- Heart	- Bone	
Flower Plants	+ Hairclip	+ Fish (P)	<u>UBA-1</u>
- Skeleton	+ Kidney	- Gun	- Leg
- Spider	+ Lungs	+ Leg, animal,	+ Penis
	- Mouth	human	
<u>CBA-1</u>	- Ribs	- Snake	<u>UBA-2</u>
- Animal	+ Ribbon (P)	- Tree	+ Breast
+ Birds Two	- Testicles		
- Clouds		<u>CBA-7</u>	
- Dog	<u>CBA-4</u>	+ Head, Bird	
- Elephant	- Backbone	+ Head, Human	
+ Humans, Two	+ Basket		
(P)	+ Bones	<u>CBA-8</u>	
- Insect	+ Butterfly	- Animal	
+ Monkeys	- Crab	+ Bird	
+ Skeleton,	- Face	- Fish	
Human	- Foot	+ Human,	
+ Toys	+ Hat	Upper	
- Trees	- Heart	part	
	+ Hip	- Insect	
<u>CBA-2</u>	- Kidney		
- Arm	- Lungs	<u>CBA-9</u>	
+ Bird	+ Pelvis	+ Bird	
+ Dog	- Ribs	+ Cartoon	
+ Fire	- Stomach	+ Dog	
- Fish	+ Trees	+ Doll	
- Lung	+ Vase, Flower	+ Human (P)	
+ Monkey	- Vagina	- Insect	
+ Parrot (↓)		+ Monkey	
- Rabbit		- Rabbit	
+ Squirrel			
TBA= W; CBA=D; UBA=Dd			

**Plate-IV: Common Blot Areas (CBA)
and Uncommon Blot Areas (UBA)**



CBA-1



CBA-5



UBA-2



CBA-2



CBA-6



UBA-3



CBA-3



CBA-7



UBA-4



CBA-4



UBA-1

TBA= W; CBA=D; UBA=Dd

**Plate-IV: Sample Shape Appropriate (+)
and Shape Inappropriate (-) Responses**

<u>TBA</u>	<u>CBA-1</u>	<u>CBA-4</u>	<u>CBA-7</u>
- Amoeba	- Animal	+ Bird	+ Animal
+ Animal,	- Bat	- Ear	+ Bat (↓)
Massive	+ Bush	- Fish	- Crab
Furry (P)	+ Candle	+ Lizard	- Face
+ Bat	+ Caterpillar	- Penis	+ Giant
- Backbone	- Crow	+ Snake (P)	- Head
- Bee	- Fish		+ Human
+ Bird	- Flower	<u>CBA-5</u>	+ Mask
+ Butterfly	+ Hand	+ Bone,	+ Monster
+ Cactus	- Human	Skeletal	
+ Cloud	+ Insect	+ Column	<u>UBA-1</u>
- Crab	- Penis	- Fish	+ Hill
- Elephant	- Snake	- Insect	+ Cap
- Fly	+ Spinal Cord	+ Pole	
- Frog	+ Tree Trunk	+ River	<u>UBA-3</u>
+ Hide (P)		+ Rocket	- Face
+ Human	<u>CBA-2</u>	+ Spinal Cord	- Tongue
dressed in	+ Bear	+ Tree	+ 'Lavang'
animal skin (P)	+ Dog	+ Vertebrae	+ Nail
- Insect	+ Foot (P)		
+ Leaf	+ Pig (←)	<u>CBA-6</u>	
+ Mountain	+ Shoe (P)	+ Boot (P)	
- Owl		- Face	
+ Pelt (P)	<u>CBA-3</u>	+ Foot	<u>UBA-4</u>
- Rib	+ Butterfly	+ Leg, Human	- Bird
+ Rug (P)	- Face	(P)	+ Toe
- Rocket	+ Head, Animal		+ Front part of
+ Sea Animal	- Head, Human		a shoe
+ Skin, Animal	- Insect		
(P)			
+ Tree			
- Turtle			
TBA= W; CBA=D; UBA=Dd			

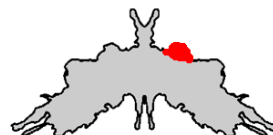
**Plate-V: Common Blot Areas (CBA)
and Uncommon Blot Areas (UBA)**



CBA-1



CBA-4



UBA-1



CBA-2



CBA-5



UBA-2



CBA-3

TBA= W; CBA=D; UBA=Dd

**Plate-V: Sample Shape Appropriate (+)
and Shape Inappropriate (-) Responses**

TBA

- + Airplane
- + Bat (P)
- Bee
- + Beetle
- + Bird
- + Butterfly (P)
- + Cloud
- + Crow
- Flag
- + Insect, Winged
- Lungs
- + Moth (P)
- Owl
- Spider

CBA-1

- Arrow
- Gate
- + Leg, animal, human (P)
- Nose

CBA-2

- + Animal
- Crab
- + Human, in reclining position
- Insect
- Kangaroo
- Leaf
- Leg
- + Rock
- Tree

CBA-3

- Bird
- + Devil
- + Face, Animal
- Face, Human
- Tuning Fork
- Vase

CBA-4

- + Animal
- + Demon
- Fish
- + Human
- + Rabbit
- Skeleton
- Tree

CBA-5

- + Feet, Animal, human
- Head
- Insect
- + Legs, Bird, Animal, Human
- Vagina

UBA-1

- Breast
- + Human Face
- + Mountains

UBA-2

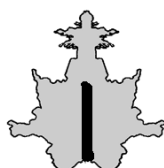
- Breast
- + Mountains

TBA= W; CBA=D; UBA=Dd

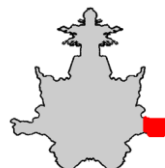
**Plate-VI: Common Blot Areas (CBA)
and Uncommon Blot Areas (UBA)**



CBA-1



CBA-4



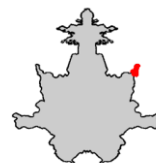
UBA-2



CBA-2



CBA-5



UBA-3



CBA-3



UBA-1



UBA-4

TBA= W; CBA=D; UBA=Dd

**Plate-VI: Sample Shape Appropriate (+)
and Shape Inappropriate (-) Responses**

TBA

- + Animal Pelt or Skin, Hide, Rug (P)
- Bat
- Bear
- Bee
- Beetle
- + Bird
- Butterfly
- Camel
- + Candle with base
- Crab
- Crane
- Crow
- Crown
- Doll
- Fish
- Genitals, Male
- + Insect
- Mosquito
- + Tree
- + Tortoise

CBA-1

- + Animal Pelt or Skin, Hide, Rug (P)
- Chest
- Fish
- + Leaf
- Locket
- + Tortoise

CBA-2

- + Bullet
- Fish
- Human
- + Missile
- + Pole
- + Reptile
- + Rocket

CBA-3

- + Bird
- + Crucifix
- + Emblem
- Face
- + Flower
- + Human
- + Insect
- + Lamp
- Leaf
- + Statue

CBA-4

- Arrow
- + Backbone
- + Canal
- + Candle
- Human
- Penis
- + River
- + Snake
- + Spinal Cord
- + Tree Trunk

CBA-5

- + Flower
- + Fountain
- + Insect
- + Plant
- + Rocket
- Spinal Cord
- Turtle

UBA-1

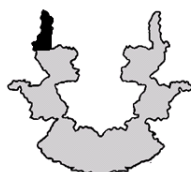
- + Penis, Front portion of
- + Mouth, Reptile

TBA= W; CBA=D; UBA=Dd

**Plate-VII: Common Blot Areas (CBA)
and Uncommon Blot Areas (UBA)**



CBA-1



CBA-5



UBA-1



CBA-2



CBA-6



UBA-2



CBA-3



CBA-7



CBA-4



CBA-8

TBA= W; CBA=D; UBA=Dd

**Plate-VII: Sample Shape Appropriate (+)
and Shape Inappropriate (-) Responses**

TBA

+ Animals
- Beard
- Bird
- Butterfly
- Chair
+ Cloud
- Dogs
+ Dolls
- Frog
+ Gate
+ Humans, two
(P)
- Insect
- Leaf
- Mouth
+ Necklace
+ Statue

CBA-1

- Bird
+ Bust (P)
- Cat
+ Cloud
+ Face, Human
(P)
- Fish
+ Head, Human
(P)
- Insect
+ Rabbit
+ Statue, Human
(P)
- Tree

CBA-2

+ Angel
- Bird
+ Cat
+ Cloud
- Cow
- Donkey
+ Dwarf
- Fish
- Frog
- Head
- Horse
+ Human face
+ Rabbit
+ Statue
+ Toy

CBA-3

- Beard
- Cap
- Cup
- Dog
- Hand
+ Head,
Animal
+ Head,
Human
- Insect
+ Thumb

CBA-4

- Airplane
+ Bat
+ Bird
+ Bow
+ Butterfly
- Buttocks
+ Cloud
+ Hill
- Human
+ Pelvis
- Vagina

CBA-5

- Human
+ Plait
+ Thumb

CBA-6

+ Canal
+ Door
+ Doll
+ Fountain
+ Human
- Insect
- Penis
+ River
- Tower
+ Vagina
- Worm

CBA-7

+ Bust (P)
+ Face, Human
(P)
+ Head, Human
(P)
+ Statue,
Human
(P)

CBA-8

+ Bowl
- Cloud
+ Head, Human
- Tree
+ Vase

UBA-1

- Arm
- Bird
- Face
+ Finger
- Head, Animal
- Penis
+ Thumb

UBA-2

- Animal
+ Head, Animal

TBA= W; CBA=D; UBA=Dd

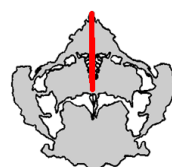
**Plate-VIII: Common Blot Areas (CBA)
and Uncommon Blot Areas (UBA)**



CBA-1



CBA-5



UBA-1



CBA-2



CBA-6



UBA-2



CBA-3



CBA-7



UBA-3



CBA-4



CBA-8

TBA= W; CBA=D; UBA=Dd

**Plate-VIII: Sample Shape Appropriate (+)
and Shape Inappropriate (-) Responses**

TBA

- + Art, Modern
- Bat
- Bird
- Butterfly
- Crab
- + Crown
- + Design
- + Emblem
- Explosion
- Face
- + Flower
- + Fountain
- + Garden
- Heart
- Lung(s)
- + Ornament
- Tree

CBA-1

- + Animal (P)
- + Bear (P)
- + Beavers (P)
- Bird
- + Cub (P)
- Fish
- Flower
- + Fox
- Frog
- Human
- Insect
- + Lizard
- + Muskrats (P)
- + Mice (P)
- + Rodents (P)
- + Squirrel (P)
- + Polar (P)
- + Rat (P)

CBA-2

- Animal
- Bat
- + Butterfly
- Buttocks
- Chest
- Emblem
- + Flower
- + Frock
- Frog
- Hat
- Heart
- Lungs
- Stomach

CBA-3

- Badge
- + Bone
- Structure (P)
- Cave
- Door
- Face
- + Fish
- backbone (P)
- Head
- + Rib Cage (P)
- + Spinal
- Column (P)
- + Thorax (P)

CBA-4

- + Bat
- + Bush (P)
- + Crown
- Face
- Fish
- Frog
- + Mountain
- Octopus
- + Tree (P)
- + Umbrella

CBA-5

- + Bird
- + Blouse
- Bone
- + Butterfly
- Chest
- + Flag
- Flower
- + Lake
- + Leaf
- Lungs
- Skull

CBA-6

- Bird
- + Bush
- + Christmas Tree
- Crab
- Face
- + Flower
- + Mountain
- + Tree

CBA-7

- Animal
- Bird
- + Blood
- Buttocks
- Chest
- Face
- Flesh
- Head
- + Mountain
- + Rocks
- + Tree

CBA-8

- + Butterfly
- Face
- Human

UBA-1

- + Bone
- Esophagus
- + River
- + Spinal Cord
- + Sticks

UBA-2

- + Antennae
- + Arrow
- Birds
- + Beak
- + Fingers
- Teeth
- + Tree

UBA-3

- + Dog (←)

TBA= W; CBA=D; UBA=Dd

**Plate-IX: Common Blot Areas (CBA)
and Uncommon Blot Areas (UBA)**



CBA-1



CBA-4



CBA-7



CBA-2



CBA-5



CBA-3



CBA-6

TBA= W; CBA=D; UBA=Dd

**Plate-IX: Sample Shape Appropriate (+)
and Shape Inappropriate (-) Responses**

TBA

- Cloud
- Face
- + Fire
- + Flower
- + Fountain
- + Garden
- Human
- Insect
- Leaf
- + Mask
- + Mountain
- + Plant
- Skull
- + Tree
- + Vase

CBA-1

- + Bird
- + Clown (P)
- Dog
- + Dwarf (P)
- + Fire
- + Fireman (P)
- Fish
- + Flower
- + Ghost (P)
- + Hill
- + Human Figure (P)
- Insect
- Lungs
- + Santa Clause (P)
- Tree
- + Witch (P)

CBA-2

- Bird
- Butterfly
- Cat
- Dog
- Elephant
- Frog
- + Grass
- Heart
- Human
- Lungs
- Tree

CBA-3

- + Apple
- + Ball
- Eye
- Fish
- + Face, Human (P)
- + Flower
- + Head, Human (←)
- (P)
- Turtle

CBA-4

- + Animal
- + Bone
- + Candle
- Human
- Insect
- + Missile
- Penis
- + Reptile
- Tree

CBA-5

- Animal
- + Cloud
- + Flower
- + Fountain
- Liver
- + Rock
- + Tree
- + Umbrella (↓)

CBA-6

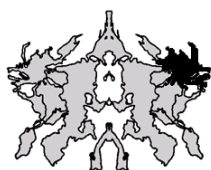
- + Bottle
- Chest
- + Hourglass
- Human
- + Light Bulb
- Skull
- + Vase
- + Waterfall
- Womb

CBA-7

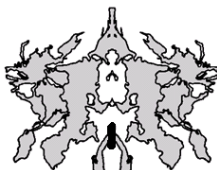
- Animal
- + Fire
- + Leaves
- Monster
- Tree
- X-ray

TBA= W; CBA=D; UBA=Dd

**Plate-X: Common Blot Areas (CBA)
and Uncommon Blot Areas (UBA)**



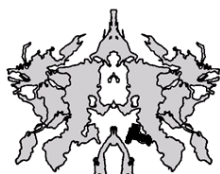
CBA-1



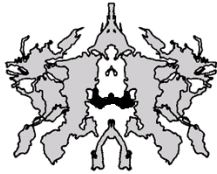
CBA-6



CBA-11



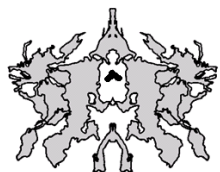
CBA-2



CBA-7



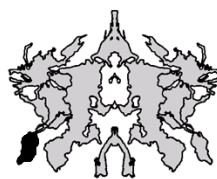
CBA-12



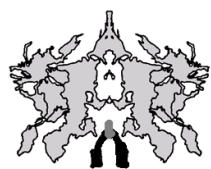
CBA-3



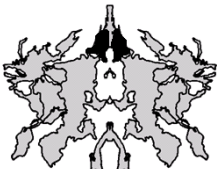
CBA-8



CBA-13



CBA-4



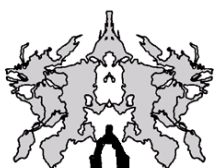
CBA-9



CBA-14



CBA-5



CBA-10



CBA-15

TBA= W; CBA=D; UBA=Dd

**Plate-X: Sample Shape Appropriate (+)
and Shape Inappropriate (-) Responses**

TBA

+ Animals, Sea
+ Aquarium
+ Art, Modren
+ Flower
Garden
+ Insects
+ Underwater
Scene

CBA-1

+ Amoeba
- Animal
+ Crab (P)
+ Insect
+ Lobster (P)
- Lungs
+ Octopus
+ Scorpion
+ Spider (P)

CBA-2

+ Cat
+ Dog (P)
+ Flower
+ Frog
+ Insect
+ Lion (P)
- Monkey

CBA-3

- Bird
- Crab
- Flower
- Testicles

CBA-4

- Arm
+ Caterpillar
- Fish
+ Sea Horse
+ Snake
- Tree
- Worm

CBA-5

+ Angel
+ Caterpillar
+ Coral
- De vil
+ Fire
- Hair
- Head
+ Human
+ Lungs
- Tree
+ Worm

CBA-6

+ Angel
- Bug
+ Bunny (P)
- Face
+ Head,
Rabbit (P)
+ Head,
Animal
- Head,
Human
+ Human (↓)
- Insect
+ Tooth (↓)

CBA-7

+ Birds
- Breasts
- Eyeglasses
+ Humans (↓)
+ Insect
- Lungs
- Nose
- Ovaries

CBA-8

+ Animal
- Ant
- Bird
+ Cockroach
- Crab

+ Deer (P)
- Dog
- Fish
- Frog
+ Grasshopper
+ Hare (P)
- Human
- Kidney

CBA-9

+ Ant
+ Bee
+ Beetle
+ Blood Spot
+ Buffalo
+ Bug
+ Bull
- Cat
- Face
- Fish
- Human
+ Insect
- Sea Animal
- Spider

CBA-10

- Animal
+ Butterfly
- Fish
- Flower
- Head
+ Insect
+ Plas
+ Rinchi
+ Snake

CBA-11

- Animal
- Broom
+ Candle, With
holder
- Face

- Human
+ Missile
+ Spaceship

CBA-12

- Animal
+ Bug
+ Bull
+ Cow
+ Dog
- Fish
+ Goat
+ Grasshopper
+ Insect
+ Leaf
+ Plant, Sea

CBA-13

+ Animal
+ Bear
- Bird
+ Dog
- Tree

CBA-14

- Animal
+ Bone
+ Candle
+ Chimney
- Head
- Human
- Knife
+ Missile
+ Rocket
+ Spinal Cord

CBA-15

+ Bird
+ Butterfly
+ Flower
- Head

TBA= W; CBA=D; UBA=Dd

CHAPTER-5: MOST FREQUENT (POPULAR) RESPONSES

Most Frequent Responses:

1. The most frequent responses are also called Popular Responses and scored as P in the P column of the response sheet.
2. The decision of most frequent responses on each inkblot has been made statistically.
3. Rorschach examiner does not have liberty to score most frequent responses arbitrarily.
4. The most frequent responses may be taken as fixed and locked. No departure should be made in their scoring. That is, if the blot area and the perceived object matches the list of popular responses, then only the most frequent response should be scored. If there is any deviation in blot area or the perceived object, most frequent response should not be scored unless indicated.
5. All most frequent responses are shape appropriate. No shape inappropriate response should be scored as most frequent response. **Caution:** if you erroneously score a response as shape inappropriate you may miss the scoring of most frequent response. In the learning phase you may frequently refer to the list of popular responses.
6. Most frequent responses are scored on either Total Blot Areas or Common Blot Areas. There is no most frequent response for any Uncommon Blot Area.
7. When you score a response as shape appropriate (+) irrespective of the nature of object attribute, immediately look into the table if the response is a most frequent one.
8. All shape appropriate (+) responses are not most frequent responses. Only a few shape appropriate (+) responses constitute the most frequent (P) responses.
9. After scoring a few Rorschach protocols you would remember many of the listed most common responses.
10. The Rorschach Inkblots are symmetrical. A perceived object may include both lateral sides (pair response) or any of the lateral side. For example, on Card-VIII, the response may be

two bears to both lateral sides (pair response) or a single bear to any of the lateral sides. In all such instances, P would be scored. Therefore, most frequent responses are scored irrespective of laterality of blot area.

11. Never score more than one most frequent (P) response for a single blot area of a card.
12. In a few cards most frequent responses are listed as '**OR**'. In such instances score one and only one of the alternatives as most frequent response (P) even if both the alternatives are produced in the response phase by the subject.
13. Be conservative in the scoring of most frequent responses (P).
14. Beck et al. (1961) and Asthana (1971) have prepared a list of most frequent responses. Pershad and Parekh (2001) has presented separate listing for most frequent responses (P) obtained by Beck et al and Asthana.
15. A pooled list of most frequent responses (P) derived from the publication of Pershad and Parekh (2001) is presented in this manual.

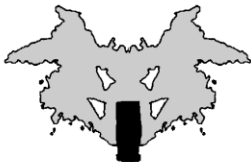
List of Most Frequent Responses – P	
The Shaded Blot Areas	P Responses
 Plate-I: Total Blot Area	Bat / Butterfly / Moth/ Any other winged creature
 Plate-I: The Lower Central Portion	Human Form
 Plate-I: Complete Central Black Portion	Human Form
 Plate-I: Any Lateral side of the Blot	Map of India

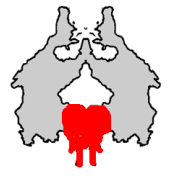
 Plate-II: Total Blot Area	Two Human
 Plate-II: Any Lateral Half	Human Form
 Plate-II: Any Side Black Area	Dog / Bear
 Plate-II: Lower Red Area	Butterfly / Moth
 Plate-III: Complete Black Area	Two human / Dolls / Statues

 Plate-III: Any Lateral Side of Black Area	Human Form
 Plate-III: Middle Red Area	Bowtie / Ribbon / Butterfly
 Plate-III: Lower Side Area	Fish
 Plate-III: Lower Central Side Black Area	Two Trees
 Plate-IV: Total Blot Area	Animal Skin like hide, pelt, rug/ Human dressed in animal skin / Massive Furry Animal

 Plate-IV: Any Side Lower Gray Area at the Extreme End	Human Foot / Shoe
 Plate-IV: Any Side Lower Gray and Black Area	Boot / Human Leg
 Plate-IV: Any Upper Lateral Portion	Snake
 Plate-V: Total Blot Area	Bat / Butterfly /Moth
 Plate-V: Upper Projections of Any Lateral Side at the Extreme End	Leg, Human or Animal



Plate-VI: Total Blot Area

Or



Plate-VI: Complete Lower Area

Animal hide, Pelt, Skin, Rug



Plate-VII: Top Area of Any Side

OR



Plate-VII: Top and Middle Area of Any Lateral Side

OR

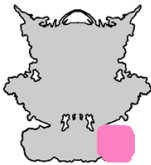
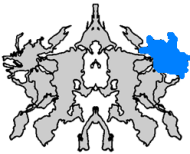
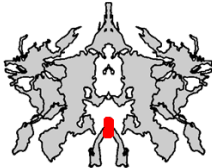
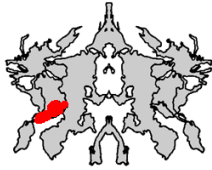


Plate-VII: Any Lateral Side

OR

Human Head / Human Faces
/ Human Statue/ / Busts/ W
as two human

 Plate-VII: Total Blot Area	
 Plate-VIII: Any Lateral End	Animals/ Rat / Mice / Bear / Cub / Polar / Squirrels / Rodent / Beavers / Muskrats
 Plate-VIII: The Upper Middle Central Portion, May Include White Background Area	Skeletal Form: Ribs / Bones / Thorax / Spinal Column / Fish Backbone
 Plate-VIII: Upper Grayish Blue portion	Tree / Bush
 Plate-IX: Any Lateral Orange Area	Human Figure/ Clown/ Dwarf / Fireman / Ghost / Santa clause / Witch

 Plate-IX: Extreme End of Lower Pink Area	Human Head / Human Face
 Plate-X: Any Lateral Blue Area	Crab / Lobster / Spider
 Plate-X: Any Inside Yellow Area	Dog / Lion
 Plate-X: Top Area of Lower Central Portion	Rabbits Head / Bunny
 Pate-X: Side Gray Area	Hare / Deer

CHAPTER-6: OBJECT CATEGORIES (CONTENT)

1. Every perceived object would fall in one or the other object category. The relevant category of the perceived object is recorded in C (Content) column of the response sheet.
2. The list of object categories may be exhaustive. Not all object categories have equal interpretative significance. There are a few object categories which are of maximum importance. Hence, you could score only the important object categories and skip the other ones by putting a dash mark in the object category column of the response sheet.
3. The important object categories are listed here. Code all applicable object categories for a percept.

A. Most Important Object Categories:

- a. **Complete Human (H):** All complete human beings are coded as H. Mythological entities in human forms are also coded here. Examples: two ladies, two persons dancing, a baby, fairy, ghost
- b. **Part Human (Hd):** If the perceived object is any external part of the body then it is coded as Hd. Examples, human faces, leg of human being
- c. **Complete animal (A):** All complete animals, birds, reptiles are coded as A. For examples: bat, butterfly, rabbit, bears, snake, gorilla
- d. **Part Animal (Ad):** All external parts of animals are coded as Ad. For example: wings of a bat, face of a monkey
- e. **Internal Organs (An):** All internal organs of living beings are coded as An. For example, lungs, kidney, ribs

B. Less Important Object Categories:

- f. **Art:** Art objects, decorations, drawings, paintings, caricatures, cartoons, statues.
- g. **Architecture (Arch):** All building structures
- h. **Blood (Bl):** All blood responses are coded as Bl.
- i. **Botany (Bt):** All plants are coded as Bt.
- j. **Clouds (Cl):** All cloud responses are coded as Cl.

- k. **Clothing (Cg):** All forms of clothes
- l. **Flowers (Fl):** All flowers are coded as Fl.
- m. **Food (Fd)** Edible food items
- n. **Geography (Ge):** Maps, territorial drawings
- o. **Household (Hh):** Household and domestic items
- p. **Landscape (Ls):** Mountains, rocks, islands and underwater scenes
- q. **Religious (Rl):** All religious percepts like temple, god, goddess are scored as Rl.
- r. **Science (Sc):** Objects related to science or science-fiction. The objects which are the products of technology and industry are also included. Car, Airplane, Computer.
- s. **Sex (Sex):** All sex organs whether internal or external are coded as Sex. For example, breasts, uterus.
- t. **Travel (Tr):** All means of travel
- u. **Misc (Ms):** Place all remaining items under this category

CHAPTER-7: QUALITATIVE SCORING

Qualitative Scoring: Following qualitative indices are explored and mentioned in Scoring Sheet of a Rorschach protocol.

1. **Approach:** Statistical norms have been developed for each of the Blot Areas i.e. W, D and Dd. In a protocol of an average healthy person, there is a balance in the production of W, D and Dd responses. In some instances it is found that one or the other Blot Area dominates the protocol. It may be any of the areas: Total Blot Area, Common Blot Area or Uncommon Blot Area. If all the areas are selected as per the norms then it is scored as Balanced Approach. If there is accentuation of any Blot Area then the dominance of that blot area is scored. For example, if W is more than average, then scoring would be W dominated approach. For higher D areas the scoring would be D dominated approach and for more than average Dd responses the approach would be scored as Dd dominated approach.
2. **Absence of M Responses:** If a protocol completely lacks human action responses, then this indice is to be scored.
3. **Absence of Popular Responses:** If a protocol completely lacks most frequent responses, then this indice is to be scored.
4. **Absence of Chromatic Color:** If a protocol is devoid of color attribute then this indice is to be scored.
5. **Ancient Objects:** When a subject perceives any object of an ancient era, this indice is to be scored. For example, ancient statue, ancient picture, ancient man, a rock of a pre-historic period.
6. **Automatic Phrases:** When a subject unconsciously repeats a phrase during response phase, this indice is scored. The repetition of a phrase should be observed for at least three times in a protocol to score this indice. For example, Card-I: a bird, *go now*, Card-III: Trees, *go now*, Card-VIII: rat, *go now*.
7. **Awareness of Inadequate Percept (Impotence):** In impotence, a subject is aware that his perception is inaccurate/inadequate wishes to improve it but he is not able to succeed in improving the percept. This should be reflected in at least three responses. For Example, Card-I:

dog (**Subject:** it does not look like a dog); Card-III: Buffalo (**Subject:** it does not seem to be a buffalo).

8. **Black & White and Chromatic Color Attribute in a Single Response (Y+C):** if both the attributes – Black & White (Y) and Chromatic Color (C) is present in a single response then this indice is to be scored. For example, on Card-II: a black bear with red blood spots; On Card-III: Modern art filled with red and black colors
9. **Black & White Response on Polychromatic Cards:** If a subject produces a Black & White Response on a polychromatic card, this indice is scored. Card-X: black gun; Card-VIII: gray bush.
10. **Bizarre Response:** When a perceived object is grossly unconventional or unusual, this indice is scored. For example, a flying rat, butterfly eating a fish.
11. **C Responses on Achromatic Cards:** If a subject produces a chromatic color response on the achromatic card, this indice is scored. For example, Card-I: Red Blood Spot, Card-VII: Red flowers and their base.
12. **Card Description:** In card description, a subject instead of producing a scorable response, just describes the inkblot. For example, there is a line in the middle, there are light and dark areas, here is something at the top. While scoring the Card Description, mention the card numbers in the parenthesis on which card description is observed. Card Description may be found along with color naming for example, on card-IX: there is an orange color at the top, below it is the green color and at the bottom pink color, there is a line in the middle.
13. **Card Rejection:** If a card is returned by a subject without producing any scorable response, then it is scored as Card Rejection. The card may be rejected within seconds or after spending some time on the card. When scoring the Card Rejection you should also mention the number of the card. For example, Card Rejection-V, IX.
14. **Color Naming:** When name of the color is mentioned without any scorable response. For example, red color, blue color, yellow color, green color, black color, white color, orange color. (Red blood, black cloud, yellow flower would not be scored as Color naming because all of these responses contain objects which can be scored quantitatively). For the purposes of color naming, the

naming of achromatic color is included. Quantitative scoring is not done for color naming.

15. **Confabulation:** This is also called as DW or DdW response. When a subject attends to a common blot area and extends the percept to the total blot area, DW is scored. Similarly, when a subject attends to an uncommon blot area and extends the percept to the total blot area DdW is scored. All confabulatory responses are to be marked as shape inappropriate (-). For example, on Card-VI: Total Blot Area is human being because here are his eyes. When enquired in enquiry phase what else make it look like a human being, the answer is only eyes. The quantitative scoring will be DW and it will also be mentioned as Confabulation in the qualitative analysis.
16. **Contamination:** When two shape appropriate objects are combined into a single percept to which nothing in reality corresponds, contamination is scored. For example, on Card-I: two animals taking a person into the sky; Card-III: Two persons dancing and their hearts are placed outside in the middle.
17. **Complete Rejection:** When a subject rejects all ten cards this indice is scored.
18. **Difficult Enquiry:** A few subjects particularly psychotic patients would not be able to give proper enquiry of the responses to the extent that they totally refuse to co-operate or fail to provide details of the responses such as blot areas and attributes. Difficult enquiry is also scored when a subject provide the details of the blot areas/attributes after much persuasion.
19. **Delayed Reaction Time:** When a subject takes longer time to respond to the cards, this indice is scored. For reference you may consider scoring this indice when RT_1 is more than 60 seconds on seven or more cards.
20. **Denial of Chromatic Colors:** There are some objects which inherently have colors in it, but a subject does not attribute colors to these objects. For example, flowers, paintings.
21. **Disease Percepts:** If a response is the name of a disease then it is scored as disease percept. For example, fever, TB, Cancer, HIV.
22. **Duplicate Responses:** Inspect the entire scoring for every response. If you find identical scoring for two or more responses on a card, delete the duplicates and score this

indice. For Example, Card-II: D1 F+ A; D1 F+ A. Card-III: D1 M H P; D1 M H P. Do not consider duplicate scoring for quantitative analysis.

23. **Emphasis on Symmetry:** When a subject points out to symmetry, this indice is to be scored. For example, this is symmetrical, it is same on both sides.
24. **Enumeration:** When a subject counts the blot areas and do not offer any scorable response. For example, 1, 2, 3, 4.
25. **Excessive Card Turning:** If a subject turns the card frequently before producing an association to inkblots, it is scored as Excessive Card Turning. There is no objective criteria for it. However, for reference you may see if five or more card turning is present on three or more cards to score this indice.
26. **Pure Eye Responses:** When a subject perceives *only Eyes* then Eye Responses is scored. When the response is face or complete body and eye is mentioned as part of this response in response phase or enquiry phase this indice is not scored. The response should be pure eye for its scoring.
27. **Expression of Doubts:** When a subject expresses doubts on his associations this indice is to be scored. For example, this is a monkey, but I have a doubt regarding it.
28. **Failure of P on Card-V:** If a subject fails to produce popular response on Card-V, this indice is scored. Card-V elicits a popular response most frequently; the absence of it is significant.
29. **Hdx or Adx Responses:** When a particular blot area commonly elicits a complete animal or human response, the subject does not perceive the complete human or animal on such blot area but perceives only a part human or animal then Hdx or Adx response as applicable is scored. For example, Card-III commonly elicits a human response, client perceives only face of human being (Hdx), Card-V: bat is the most common response, but the subject perceives only bat's wing (Adx).
30. **Hidden Objects:** When a subject perceives hidden objects like a person hiding behind a tree, a person hiding his hands, then Hidden Objects is scored.
31. **Looking at the Back of the Card:** When a subject turns the card to the back and examines the card in that position, this indice is to be scored. A subject may look at the back of the

card very quickly or may spend considerable time in that position and try to read the publisher details.

32. **M in Animal Percepts:** In the object attributes, if you find that a human purpose, emotion or action is present in an animal percept, then this indice is to be scored. For example, a laughing dog, two bird talking.
33. **Methodical, Irregular or Confused Sequence:** According to the Gestalt principles, the perception proceeds from Whole to Part. In Rorschach it is translated into the following sequence: W-> D-> Dd. That is, when presented an inkblot, a subject should first produce a W response, then D response and finally Dd response. The deviations in this sequence may be present in the Rorschach protocol of a subject in any of the following form: D-> W-> Dd; Dd->W-> D; Dd->D->W. To score this indice, the scoring is inspected card to card. If on all the cards, the sequence of responses is W->D->Dd then it is scored as Methodical Sequence. If there is deviation in the sequence on one card, it is labeled as Regular Sequence, if deviation is present in two-three cards; it is scored as irregular sequence. Finally, if deviation is present in more than three cards, Confused Sequence is scored.
34. **Mono-Response Protocol:** If a subject perceives one and only one and the same object on all plates of the Rorschach, mono-response protocol is scored. For example, Bat on every cards, no other response.
35. **Multiple Card Rejection:** If a subject rejects three or more cards, Multiple Card Rejection is scored.
36. **Narrow Content Range:** If the number of object categories in a protocol is three or less, then this indice is scored.
37. **New Responses in Enquiry:** If a subject produces totally new responses in the enquiry phase, then such responses are not scored quantitatively. Instead they are scored qualitatively as New Responses in Enquiry. You should write new responses in the enquiry column in parentheses. You may mention the number of new responses produced in the enquiry phase. For example, New Responses in Enquiry (7). It would mean that the subject produced seven new responses in the enquiry phase.
38. **Non-productive Handling of Cards:** This indice is scored when a subject retains the cards for sufficient time i.e. two or more minutes and do not produce any scorable response

to the cards. Remember, two scorings would be applicable for such instances, Card Rejection and Non-productive Handling of the Cards.

39. **Odd Card Positions (Edging):** If a subject holds the cards at the corner and looks at it from the distance or holds the card in odd positions during the responses phase, then this indice is to be scored. If a subject rotates a card at least three times in circular manner, this may be scored as spiral card position.
40. **Pathological Anatomy:** If the perceived object is an internal organ, and the subject perceives it as mutilated, pathological, destroyed, disfigured then this indice is to be scored. For example, diseased lungs, damaged kidney.
41. **Perplexity:** When a subject is bewildered manifested by the remarks such as 'what it could be' 'I can not think', 'have I given a correct response?' this indice is scored.
42. **Personal Reference:** When a subject relates the perceived object to self then this indice is scored. For example, my uncle's face, my own image, this is the coat very similar to the one I had.
43. **Precision Alternatives:** If a subject produces two or more responses to the same blot area by saying "or" then precision alternative is to be scored. For example, top red area of Card-II: two human beings or two caps.
44. **Pseudo-human Percepts:** if a protocol contains three or more pseudo-human percepts like ghosts, witch, monster, demon this indice is to be scored.
45. **PO Response:** When a subject in the enquiry says that the perceived object looks like that because of its position then Position Response is scored. For example, kidney because the location of the kidney is the same in the body; Heart because heart lies exactly at the same place in the body. Instead of the attributes, the position of the object determines the response. In object attribute column of the response sheet write PO as the attribute. F, C, Y, M, T,V are not scored for PO responses.
46. **Qualifiers:** When a subject engages in self-criticisms like 'I am not educated' 'I have not seen these figures before', 'I have not read biology', 'my mind is weak', this indice is to be scored.

47. **Quick Reaction Time:** When a subject responds faster to the cards, this indice is scored. For reference you may consider scoring this indice when RT_1 is less than five seconds on seven or more cards.
48. **Repetitive Responses (Perseveration):** Repetition of the same response to three or more consecutive cards without any intervening responses is scored as Perseveration. For example, Card-I: bat, Card-II: bat, Card-III: bat. There was no other response in between these bat responses. (Bat, Dog, Bat, Bat, monkey would not be scored as perseveration)
49. **Repetition of the Same Response on a Card:** If a subject repeats the same response to the same blot area on a card because he forgets that he has already given the response, this indice is to be scored. There must be at least one intervening response between repetitions. For example, Card-I: D4 a lady; W as bat; D4 a lady.
50. **Remarks:** when a subject gives a remark to the inkblots then this indice is to be scored. For example, this is a nice drawing, this image is not attractive, it is a beautiful picture.
51. **Response Rejection:** If a subject produced a response in the response phase but denies it in the enquiry phase then Response Rejection is scored. The subject may say that he did not give this response; he is not able to see this object now.
52. **Response Replacement:** If a subject produced a response in the response phase, denies it in the enquiry phase, instead offer a new response to the card, then Response Replacement is scored. For example, On Card-II: dog. In enquiry, I have not told dog; there are two persons instead.
53. **Shock:** Shock is usually scored as shading shock or color shock. If the shock is present on any achromatic cards it would be scored as shading shock. If the shock is observed on any of the chromatic cards it would be scored as color shock. The evidence of shock may be found both quantitatively and qualitatively. For quantitative evidence, the scoring on each card is inspected. If followings are present then shock may be scored. Compared to other cards, if on any card, number of response is less, more shape inappropriate responses are produced, reaction time to first response is increased, failure in providing most frequent response on the card, sequence is altered, production of Hdx or Adx response and excessive card turning. The card may be rejected because of the shock. Qualitatively it may be

noted that a subject makes a negative remark on the card. For example, what an ugly card!, I do not like this color combination, please remove this card from my sight, nothing is made on this card. If 2-3 evidences are present, then shock phenomena is scored.

54. **Stereotype:** When any object category is present in 60% or more responses of a protocol then stereotype is scored. The nature of stereotype may be mentioned in the scoring.
 - A. **Anatomy Stereotype:** More than 60% internal organs in a protocol
 - B. **Body Part Stereotype:** More than 60% body parts in a protocol
 - C. **Religious Stereotype:** More than 60% religious responses in a protocol
 - D. **Sex Stereotype:** More than 60% sex responses in a protocol
55. **Variable Reaction Time:** When a protocol consists of a combination of following RT₁ quick, delayed and average, then Variable Reaction Time is scored.
56. **Variability:** When a subject produces adequate and appropriate responses to either first or second half of the test and gives poor and inadequate responses to other half, then this indice is to be scored. For instance, a subject produces adequate responses to first five cards and inadequate responses to last five cards.
57. **Vague Responses:** If you find 25% or more vague responses then score this indice. The vague responses are the responses which contain accidental shapes like fire, cloud, smoke, blood.
58. **Wide Content Range:** If the number of object categories is large (more than seven) in a protocol, wide content range is to be scored.
59. **Word Finding Difficulty:** When a subject provides narratives of an object and fails to recollect corresponding label for the object this indice is scored. For example, on Card-V: a bird that flies in the night, just I am not able to recall its name (the subject is not able to tell bat).
60. Rorschach Images have been symbolized to represent certain characteristics or conflicts. A shock or rejection on a Rorschach plate may indicate corresponding conflict. This is presented in following table:

Plate No.	Significance/Conflict
I	Adjustment to Test Situation
II	Initial Color Shock, Female Sex
III	Interpersonal Relationship
IV	Attitude Towards Male Authority, Father Card
V	Reality Card
VI	Phallic Card
VII	Mother Card
VIII	Reality Card
IX	Phobic Reaction
X	Emotional Tolerance

CHAPTER- 8: COMPUTATIONS FOR QUANTITATIVE INDICES

1. **Total Number of Responses (TR):** TR is the Total Number of scored responses. For computation of TR count the number of scored responses in L column of the response sheet. TR should never be counted in the 'response' column of the response sheet because there might be response rejections which are not reflected in the 'response' column of the sheet.
2. **Percent Total Blot Area (W%):** Count the number of responses scored as W in L column of the response sheet. Include the responses scored as Ws and confabulatory W. Compute W% through following formula: $W\% = W/TR \times 100$
3. **Percent Common Blot Area (D%):** Count the number of responses scored as D in L column of the response sheet. Include the responses scored as Ds. Compute D% through following formula: $D\% = D/TR \times 100$
4. **Percent Uncommon Blot Area (Dd%):** Count the number of responses scored as Dd in L column of the response sheet. Include the responses scored as Dds. Compute Dd% through following formula: $Dd\% = Dd/TR \times 100$
5. **Percent White Background Area (S%):** Count the number of responses scored as S (Ws, Ds, Dds) in L column of the response sheet. Compute S% through following formula: $S\% = S/TR \times 100$
6. **Percent Shape Appropriate Responses (F+%):** Count all responses scored as pure F+ in D column of the response sheet. Do not include responses scored as M, V, Y,T,CF,C.FC. Also count separately the responses scored as F- in D column of the response sheet. Compute F+% through following formula: $F+\% = F+ / (F+) + (F-) \times 100$. For example, there are 10 F+ responses and 5 F- responses in a protocol. The F+% would be $10/10+5 \times 100 = 66.67$
7. **Percent Human Action (M%):** Count all responses scored as M in the D column of the response sheet. Include both M+ and M- responses. Compute M% through following formula: $M\% = M / TR \times 100$.

8. **Percent Form-Color Responses (FC%):** Count all responses scored as FC in the D column of the response sheet. Include both FC+ and FC- responses. Do not include the responses scored as CF and C. Compute FC% through following formula: **$FC\% = FC / TR \times 100$**
9. **Percent Pure Color and Color-Form Responses (C+CF%):** Count all the responses scored as C and CF in D column of the response sheet. Include the responses scored as C+, C-, CF+ and CF-. Do not include the responses scored as FC. Compute C+CF% through following formula: **$C+CF\% = C + CF / TR \times 100$** .
10. **Percent Black and White Responses (Y%):** Count all responses scored as Y in the D column of the response sheet. Include Y+ and Y- responses. Also include YF and FY if ventured by you. Compute Y% through following formula: **$Y\% = Y / TR \times 100$** .
11. **Percent 3 Dimension Responses (V%):** Count all responses scored as V in the D column of the response sheet. Include V+ and V- responses. Also include VF and FV if ventured by you. Compute V% through following formula: **$V\% = V / TR \times 100$** .
12. **Percent Tactile Responses (T%):** Count all responses scored as T in the D column of the response sheet. Include T+ and T- responses. Also include TF and FT if ventured by you. Compute T% through following formula: **$T\% = T / TR \times 100$** .
13. **Percent Most Frequent Responses (P):** Count all responses scored as P in the P column of the response sheet and compute P% through following formula: **$P\% = P / TR \times 100$** .
14. **Percent Human Object Category (H%):** Count all responses scored as H and Hd in C column of the response sheet and compute H% through following formula: **$H\% = H + Hd / TR \times 100$** .
15. **Percent Animal Object Category (A%):** Count all responses scored as A and Ad in C column of the response sheet and compute A% through following formula: **$A\% = A + Ad / TR \times 100$** .
16. **Percent Internal Organs Object Category (An%):** Count all responses scored as An in C column of the response sheet

and compute An% through following formula: **An% = An / TR x 100**

17. **Experience Balance (EB):** Count all M responses in D column of the response sheet and give a score of 1 to each M. Count all C response in D column and give a score of 1.5 to each C, count all CF responses in D column and give a score of 1 to each CF, count all FC responses in D column and give a score of 0.5 to each FC. Now sum up the values of C, CF and FC, you will get the Csum. EB is expressed as **M : Csum**. For example there are 4 M responses, 1 C response, 2 CF responses and 2 FC responses in a protocol. The EB would be: 4 : 4.5.
18. **Affective Ratio (Afr):** Count all scored responses in L column to Plate Nos. VIII, IX and X. Also separately count all scored responses to Plate Nos. I to VII through the L column of the response sheet. Compute Afr through following formula: **Afr = Responses on Plate Nos. VIII, IX & X / Responses on Plate Nos. I to VII**. For example, there are 10 scored responses on Plate Nos. VIII, IX & X; and 20 responses on Plate Nos. I to VII. The Afr would be 10 / 20 = .50.

CHAPTER-9: INTERPRETATION

Interpretation and Symptom Value of Rorschach Indices:

1. Every Rorschach indice correspond to one or the other psychological attribute or symptoms.
2. A Rorschach indice may have multiple interpretative significance and may be found in more than one psychiatric conditions.
3. A single indice rarely point out to any psychological syndrome. It is the clustering of Rorschach indices that suggest a single or a group of psychiatric disorders.
4. Clinical judgment is required to interpret Rorschach indices in an individual.

Total Number of Responses (TR): The total number of scorable associations produced by a subject to the inkblots is indicative of productivity. A number of cognitive and emotional experiences can lead to departure from the average scored productivity. A depressed patient because of psychomotor retardation, passivity, lack of interest in environment, hopelessness, worthlessness may produce less number of scorable associations to the inkblots. Mania is the polar opposite of the depression and as such a manic patient because of flight of ideas and tendency to get involved in multiple tasks may produce excessive number of responses to the inkblots. You may even get a protocol consisting of more than one hundred responses from a manic patient. The schizophrenic patients because of stereotype thinking, withdrawal, decreased motivation, preoccupation, avolition, indifferent attitude to the environment may produce less number of responses to the inkblots. There are conditions like brain pathology, mental retardation which causes deficient cognitive functions. When asked to associate to the inkblots, a number of cognitive processes like attention, concentration, memory, judgment, fund of information, awareness of the environment are required. If there is any deficiency or impairment in these processes, the result is likely to be decreased productivity on the Rorschach. In a person of superior intelligence, because of rich experience and above average concentration, memory, judgment, fund of information, there would in general be an increase in the productivity. If a person is obsessed, because of an inner compulsion to miss nothing, such person may keep on associating to each and every part of the inkblot resulting in enhanced productivity. The feelings like anxiety, depression, disgust negatively affect productivity and the emotions like elation, euphoria increases the productivity. The productivity is also associated with the stress tolerance. The presentation of inkblots for association adds to the cognitive load of the subject. If a person has

reduced capacity to tolerate stress, this would result in decreased total number of responses.

Total Blot Area (W): The presentation of inkblots for association place cognitive demand on a subject who scans the blot area and engages in cognitive activity of analysis & synthesis; and abstract thinking. W response results when a subject is able to integrate all component parts of the inkblots. The W potentials of the Rorschach inkblots vary plate to plate. There are some plates like IV, V, VI which involve less amount of synthesizing activity because all the parts of the blot are connected to each other in a manner which make perception as a whole quite easy. On the other hand, the plates like III and X has scattered inkblot areas, the integration of which demands greater amount of synthesizing activity, hence the production of W responses on these images reflect superior intellectual functioning of an individual. The cognitive impairment in organicity, mental retardation causes reduction in W responses. The loss of abstract attitude which is typical of schizophrenia reduces the integrative cognitive activity of an individual which is reflected in decreased number of W responses. The emotional functioning which interferes in cognitive functions like anxiety, depression may also cause lesser W in a subject. The manic patients because of inflated self-esteem produce higher number of W responses. Obsessive patients because of preoccupation with minute may produce less W responses. While interpreting W responses, it is also necessary to consider the shape appropriateness of W responses. The shape inappropriate W responses do not involve the same amount of cognitive activity as the shape appropriate W responses. The studies in normal subjects have found a positive correlation of W responses with the tests of intelligence.

Common Blot Areas (D): In a typical Rorschach protocol, a preponderance of D blot areas is found. From a plethora of potential part blot areas, it is observed that there are certain areas on every inkblot which are extracted by healthy individuals frequently in a certain amount. The magnitude of D responses may be more or less than the average in a protocol. If the amount of D is more it is termed as D dominated approach. This approach may result from proportionate decrease in W or Dd. If W is compromised to produce higher D responses, it reflects an individual's cognitive style of low abstract thinking, low integrative capacity, high emphasis on obvious aspects of a situation, practical and concrete approach. If more D is produced at the expense of Dd responses it would reflect an individual's style of ignoring minor aspects of the environment and more practical orientation. Low D may result from stress experiences and poor perceptual abilities.

Uncommon Blot Areas (Dd): In an average protocol Dd areas are found to be the lowest. These are less frequently extracted areas of

the inkblots. In anxiety disorders, a patient may become preoccupied with less significant issues which is reflected in increase in Dd blot areas. Obsessive patients are known to produce higher Dd responses because of their anxiety, inner compulsion to miss nothing. The schizophrenic patients on the other hand, because of their emphasis on unnecessary aspects of the environment do produce higher Dd responses. A severe disturbance may be reflected when Dd blot areas are produced at the expense of D areas; such an individual may become restricted in fulfilling the daily demands of practical life and may have rigid thought process. Dd in general are reflective of quantity ambition, anxiety and emphasis on minute aspects of the environment.

White Background Areas (S): On Rorschach plates, inkblot portion is the figure and all white areas are the background. When presented to a subject, it is expected that a subject would associate mostly to the figures because the data from normal subjects indicate that this is the case. If a subject attends more than average to the White Background Areas, it means that he is searching for something which is normally not attended to by healthy subjects. This tendency to attend to the White Background Areas reflects negativistic, opposite, suspicious, stubborn, hostile, resistive attitude of a subject. High White Background Areas (S) may also indicate difficulty in handling anger and a potential for acting out. There are a few White Background Areas which have been found to be present in a normal sample. If a subject attends to White Background Areas other than the areas extracted by normal subjects, this indicates more marked disturbances in the subject. The accentuation of White Background Areas is generally found in paranoid, obsessive, antisocial, opposite-defiant disorder patients.

Shape Appropriate Responses (F+%): The shape determined responses occupy the largest share of object attributes in an average protocol. Such responses are interpreted in terms of their appropriateness. A number of cognitive processes are involved in the production of shape appropriate responses. When a subject receives the inkblot from the examiner in response phase, he attends and scans the blot area, recollects the pictures resembling the total or any of the part blot areas, selects one or more pictures from the memory bank to associate on the basis of some logic. It is the accuracy of matching between blot area and the perceived object which is measured by F+%. This shape accuracy may get jeopardized in a number of psychiatric problems. If a patient has little respect for the reality of the environment, there will be corresponding decrease in the magnitude of shape appropriate responses. Most of the psychotic patients because of impairment in reality testing produce low F+% responses. They fail to objectively evaluate the reality and correct their distorted perception of the reality. For psychotic patients the hallucinations and delusions are

the true reality, they believe in it, get influenced by them, their behavior is regulated by such morbid experiences. Any rational attempts to correct these experiences are not successful. There is a wide gap in the objective reality and the reality of such psychotic symptoms. Because of morbid processes the patients come to believe in the reality of these symptoms which we call impairment in reality testing. This impaired reality testing is manifested through decreased F+%. Reality testing is the function of the Ego, it is governed by reality principle. In psychotic patients, the ego strength becomes weak, F+% also indicates the Ego Strength. Low F+% are indicative of poor ego strength.

The production of shape appropriate responses involve the cognitive processes of scanning, attention, concentration, memory, judgment, perceptual filtering. The changes in cognitive functions because of brain pathology or developmental lag result in reduction in the shape appropriate responses. In such instances, low F+% are indicative of cognitive impairment. A person obsessed with accuracy may produce high shape appropriate responses. This indicates rigidity and accuracy compulsion.

Human Action (M): Rorschach images have stationary inkblots. The perception of actions in these blots is a psychological experience and thus indicates the creative energy of an individual. The individuals having superior creativity like artists, writers; advertisers may produce high shape appropriate (M+) responses. The morbid processes like hallucinations, interpersonal delusions results in production of high shape inappropriate (M-) responses. The paranoid patients with delusions of interpersonal nature tend to produce high M- responses. These M- responses are indicative of fantasy processes and idiosyncratic thinking. The manic patients because of accelerated tempo of thoughts also produce high human action responses. The human action responses may also be interpreted on the basis of the nature of plates on which they are produced. M responses on achromatic inkblots may reflect a tendency to absorb anxiety in imaginative activity. The M responses on chromatic card may be observed in manic patients. M may be present with Black & White (Y) or chromatic color (C) in a single response. The combined M.Y. attribute indicates depressive feeling. The Combination of M.C attribute in a single response reflects agitation. The attribution of Human Action to animal percepts reflects repressed aggression in the respondent. Shape appropriate Human Action responses (M+) also point out to the capacity to have motor control over feelings and impulses. Low M may indicate impulsivity and decreased capacity for empathy.

Chromatic Color (C): Color on Rorschach indicates emotional experiences mostly of pleasant type. The manic patients because of euphoric, elated mood tend to produce high color determined responses. The admixture of shape to the color responses is

indicative of the degree of control over the expression of emotions. Pure C reflects little or no control over emotional expressions. They are usually found in children, deteriorated schizophrenics and organics. Color-Form responses indicate partial control over emotional expressions. High C and CF responses in a protocol reflect impulsivity, ego-centricity, emotional lability, irritability, suggestibility and aggressiveness in a respondent. High CF responses are found in schizophrenics, manics, attention-deficit hyperactivity disorder, impulse control disorders, brain disorders. The FC responses are indicative of matured emotionality which is found in healthy adults. The FC responses indicate empathy, capacity to establish rapport with others, matured interpersonal relationships, and capacity to learn under stress. Extremely high FC on the other hand may be an indication of over compliant and dependent personality.

Experience Balance (EB): There are two sides in Experience Balance. M which is reflective of inner world and C indicates the external environment. The interpretation of EB is based on the magnitude of M and C in a protocol and their proportion. If a protocol has more than three M and more than 3 Csum value, then possible permutations are: (a) Introversive EB (b) Extratensive EB and (c) Ambiequal EB. If M side is high e.g. 5:3 then the EB would be introversive. On the other hand if C side is high e.g. 3:5 then the EB would be extratensive and if both are equal e.g. 4:4 then the EB would be ambiequal. The introversive EB indicates stable emotionality, narrow but intimate relationships, turning of feelings inward, cautious and submissive nature. Extratensive EB reflects a large social network which is relatively superficial, an externalized way of living, spontaneity and assertiveness. An Extratensive EB person directs his energy towards outside world. The extratensive EB may also be found in manic and hysterical patients. The ambiequal EB is indicative of ambivalence and may be found in Obsessive-Compulsive Disorder (OCD). If both sides of EB i.e. M:Csum is less than three but more than one then the EB is called as Constricted EB which indicates an intellectual resistance to change. The EB with a ratio of 1:1 or less is termed as Coarcted EB and indicates rigid defensive efforts.

Black & White Responses (Y): The Black & White responses are indicative of negative emotional experience of depression and anxiety. Therefore, an accentuation of Y responses in a protocol indicates sadness, depression, emotional passivity which is mostly found in depressive patients. If Y response is present on Chromatic Cards, it indicates more severe depressive feelings. From psychoanalytic perspective, mania is said to be a defense against depression. In classification schemes, mania is classified under Bipolar illness. Rorschach is said to tap even the underlying unconscious processes of an individual. It has been observed that

many manic patients produce high Y responses because of underlying depressive potentials.

3 Dimensional Responses (V): V is scored when the third dimension of depth, distance or height is present in a response. This attribute reflects feelings of inferiority and deep self-critical introspection in a subject and found mostly in neurotic patients.

Tactile Responses (T): Tactile responses are scored when the object attribute contains the touch feelings. It reflects vague and free floating anxiety in a subject. It is mostly produced by neurotic patients. High T is also an indication of intense need for affection and dependency; and oversensitivity in interpersonal relationships.

Most Frequent Responses (P): P responses are those responses which are given most frequently by the subjects. Since these responses are produced by a large number of subjects in a sample, they point out to the commonality in group thinking. There are certain behaviors in which most of the members of a society engages themselves. Also there are social and cultural norms and practices which are followed by members of a society. The shared thinking is a requisite in adherence to the social norms, team work, group activities. If a person fails to follow social norms, not able to participate in a team/group activities, it points out to the departure in his thinking which is shared by others. Most Frequent Responses (P) point out to this aspect of human behavior. An average amount of P responses indicate a subject's conformity to social norms, group thinking, recognition and respect of socio-cultural standards. The subject who produces low P responses, does not see what others easily perceive on the Rorschach images. This low magnitude of P responses point out to his inability to follow social standards, disrespect to social norms, withdrawal from social interactions and a rebellious attitude. Low P responses are generally found in severe mental illnesses like psychosis, schizophrenia, mania, organicity and developmental disabilities. In a few subject, preponderance of P responses may be obtained. The high magnitude of P responses point out to excessive adherence to social norms. Such persons may have excessive fear of departure from the safe limits of socially approved behaviors.

Human Responses: The object category Human corresponds to a subject's ability to engage in interpersonal relationships. A high human content is reflective of wide range of interest in people, high self-esteem and empathy. High H may be found in manic patients because of their tendency to get easily involved in human interaction, getting over familiar even with strangers, disinhibited social behavior. Paranoid patients having interpersonal delusions also produce high human responses because of their suspiciousness and preoccupation that others are following him, talking about him, want to harm him. Low H is an indication of withdrawal from interpersonal relationships.

Animal Responses: Animal category occupies the largest share of object category in a protocol of an average person. This is because most of the images elicit one or the other animal percept. If a person fails to produce an average amount of animal content, it reflects his restricted awareness of the environment. The excessive production of animal content reflects a narrow focus, stereotypy, guarded attitude and poverty of associative content in the subject.

Internal Organs (An): The perception of internal organs on Rorschach reflects a subject's concern with his body. The high amount of internal organ responses may point out to excessive somatic concern and the persons having somatization or somatoform disorder may produce more An responses. However, it is found that very high An responses are indicative more of stereotyped thinking instead of somatic concern and some schizophrenic patients produce an excessive amount of An responses. If such An responses are mutilated, diseased then they point out to marked anxiety, depression and distorted body image.

Affective Ratio: Affective Ratio (Afr) is computed by dividing the total number of responses on Card No. VIII, IX, X by the total number of responses on Card no. I to VII. Last three plates of Rorschach are polychromatic. Rorschach had formulated his Color-Affect hypothesis based on his observations that color on inkblots is associated with emotionality. If these polychromatic cards excite the chord of emotionality, then a subject produces more responses to these polychromatic inkblots which result in high affective ratio. The increased affective ratio indicates a subject's emotion dominated thinking, emotional excitability. The high affective ratio (more than .80) is usually observed in manic patients. Low affective ratio in contrast indicates reduced ability to experience pleasant emotions, reduced excitability. Low affective ratio (less than .40) is generally observed in Depressive patients. If a depressive patient produces high affective ratio it may be an indication of Reactive Depression.

Bizarre Response: These responses are the product of autistic thinking, fantasy process. They are found in psychotic patients.

Black & White Responses on Polychromatic Cards: Blackness on inkblots is associated with negative emotions such as anxiety and depression. A subject having these emotions tends to give more shading responses on achromatic cards. The emergence of shading responses on polychromatic cards may indicate more severe anxiety and depression in the respondent.

Card Description: When presented an inkblot, the normal expectation is that a subject would associate to the inkblots meaningfully and would perceive such objects which can be scored quantitatively. Some subjects fail to act as per this expectation and engage themselves in the description of the structure of inkblots.

This tendency of acting against expectations may point out to the underlying aggression in a subject.

Card Rejection: The card rejection indicates a marked failure to associate to the cards. There may be many reasons why a subject fails to associate to the cards. It is possible that the inkblot areas penetrate an area of unresolved conflict which a patient finds difficult to handle. Card rejection may be a sign of shading shock or color shock.

It may point out to a neurotic conflict. A detailed enquiry in neurotic patients may enable an examiner to access the conflict area in a patient. The psychotic patients because of the poverty of associative content may reject multiple cards. In such patients the rejection may not be associated with any conflict. The depressive patients have a tendency to reject the card, if an examiner pursues a little they come with adequate number of responses to the inkblots.

Confabulation: Confabulation is an indication of thinking disturbances. It is generally found in schizophrenics and organic patients.

Contamination: Contamination also point out to disturbed thought processes and generally found in schizophrenic patients.

Color Naming: Just naming the colors of inkblots like red, green, yellow, blue does not constitute as scorable responses. Color Naming point out to marked failure in producing a meaningful associative content to color blot areas. Color Naming is generally found in organics, epileptics, deteriorated schizophrenics and mentally challenged persons.

Color Denial: Since color is associated with emotionality, any denial of color on inkblots is an indication of denial or suppression of one's feelings.

Content Range: Manic patients engage in multiple activities and tend to produce wide content range. This wide content range point out to varied interests in the environment. The depressive patients because of narrowed interests in the environment produce restricted content range.

Disease Percepts: Diseases as a response to the inkblots is usually found in schizophrenic patients. Some depressed patients may also produce such responses.

Difficult Enquiry: In depressed, psychotic, organic patients it becomes difficult to conduct an adequate enquiry. These patients may take longer time to provide details solicited in the enquiry or they may keep on vaguely pointing out to the blot areas. In extreme cases they may deny most of the responses.

Emphasis on Symmetry: Rorschach inkblots are symmetrical in nature. Some subjects point out to this symmetrical nature of the inkblots. A few patients may actively engage in the process of symmetrical comparisons of both sides and point out to any asymmetry in the inkblots. This active exploration of asymmetry

reflects insecurity and fear of impulsiveness. If this pattern of asymmetry exploration is present on many cards it may point out to stereotyped thinking in a patient.

Eye Responses: Paranoid patients produce excessive part human or animal responses. The responses like teeth, eyes are indication of paranoid thinking process. These part responses should not be a component highlighted during the enquiry for complete human or animal response. Eye should be a standalone response.

Excessive Card Turning: An inquisitive subject would like to explore various blot areas to produce associative content. As such it may be a reflection of superior intellectual functions. But rapid and excessive card turning is a nervous phenomenon which point out to the anxiety in a subject. Distractibility and hyperactivity in manic patients also causes frequent card turning. An elaborate exploration of cards in various positions may reflect a subject's over concern to the environment and hyper-vigilance for cues of threat in the environment; hence it may be a reflection of paranoid thinking.

Expression of Doubts: Doubt is a symptom of OCD which is reflected in the expression of doubts on inkblot associations.

Failure of P on Card-V: Plate-V on Rorschach elicits the popular response most frequently. It has the largest frequency in a sample of normal subjects. If any patient fails to perceive the popular response on this card it becomes meaningful and indicates marked departure from the social norms.

Hdx or Adx Responses: There are some blot areas which commonly elicit a complete human or animal response. For example, Card-III, usually elicits a response containing two human figures, Card-V: Bat or Butterfly, Card-VIII: Rat. If a subject fails to notice the complete object and produce a part object it may be an indication of reduced concentration which is compromised by anxiety. This type of x responses are generally produced by anxious subjects.

Hidden Objects: A subject who is interested in searching for the hidden objects on Rorschach has marked lack of trust on others. This phenomenon is an indication of paranoid attitude.

Awareness of Inadequate Percept (Impotence): If a subject recognizes that his associations to the inkblots are inadequate and he fails to improve it, point to the inferiority feelings and reduced self-confidence.

Looking at the Back of the Card: Looking at the back of the card instead of the figure in the front; is an indication of suspiciousness in a subject. Paranoid patients usually look at the back of the cards.

Mono-response Protocol: A protocol characterized by repetition of single response to ten inkblots is an indication of severe cognitive disturbances. It is an instance of extreme stereotype. Organics, Schizophrenics and Mentally Challenged persons produce mono-response records.

Multiple Card Rejection: Psychotic patients reject many cards. Depressed patients may also have a tendency to reject many cards, but they may associate with persuasion.

New Responses in Enquiry: Manic patients because of distractibility, racing thoughts produce several new responses in enquiry.

Non-productive Handling of Cards: Psychotic patients retain a card for sufficient time, scan the inkblots and return the cards without association. It may be an indication of poverty of associative content.

Perplexity: Perplexity on Rorschach is an indication of anxiety and confusion. This can be found in a patient having marked anxiety or the person who is confused because of some pathology in the brain.

Perseveration: Perseveration is a failure in shifting the mental set. It is an example of extreme cognitive rigidity. Perseveration is a pathognomonic sign of psychiatric disorder. A patient who perseverates on inkblots may also have narrowed interests, preoccupation and stereotyped thinking. Perseveration is usually observed in schizophrenic patients and brain pathology.

Personal Reference: Personal reference or self-reference is an indication of loss of ego boundaries. It is a psychotic phenomenon. Paranoid patients frequently produce responses with personal reference.

PO Reference: PO response is an indication of disturbed thinking. PO responses are not the product of objective reasoning. Such responses are usually produced by psychotic patients.

Precision Alternatives: These responses indicate anxiety, insecurity, fear of responsibility. These responses may be found in neurotic patients.

Pseudo-human Percepts: Paranoid patients produce more pseudo-human percepts.

Qualifiers: A tendency to give excuses, criticizing oneself, underestimating one's potentials, finding one's weakness and faults in response to the demand to respond to the inkblots indicate marked feelings of inferiority, over caution, aggression turned towards self. Qualifiers are usually found in anxious and depressed patients.

Reaction Time: Reaction time is the measure of the speed with which a subject begins to associate with the inkblot stimuli. Manic patients have increased psychomotor activity, they respond faster to the inkblots. They may begin to associate even before a card is handed over to them. They have quick RT_1 . Some manic patients may respond more quickly to chromatic cards in comparison to the responding to achromatic cards. The depressed patients have retarded psychomotor activity hence they take longer time to respond to the inkblots. Their RT_1 is delayed. Schizophrenic patients have variable reaction time.

Repetition of the Same Response on a Card: Giving the same response to a card and forgetting the earlier one is an indication of amnesic process.

Sequence: Regular and Irregular sequences are normal. When the sequence becomes confused or methodical it point out to the way a subject approaches his environment. The methodical sequence is an indication of rigidity, inner compulsion for orderliness. The deviations in such subjects may elicit anxiety. The methodical sequence is found in OCD patients. The confused sequence on the other hand point out to erratic approach in dealing with the demands of the environment. Such a person is haphazard, has difficulty in planning and execution of the goal directed activities. The confused sequence may be a psychotic phenomenon and usually found in schizophrenic patients.

Shock: Any shock be it color shock or shading shock is a neurotic phenomenon. It may be found in psychotic patients who have comorbid neurotic symptoms. Shock indicates activation of a conflict area which produces anxiety to the extent that appropriate responding on the shock producing card becomes difficult for the subject. Plate-IX has the maximum shock potentials. A Rorschach examiner may be interested in exploration of the conflict area activated by the image on which shock is observed.

Stereotype: Stereotype on Rorschach is a product of cognitive rigidity, thinking disturbances, narrowed interest in surroundings. Stereotype is found in psychotic patients. Depressed patients also give stereotyped responses. Body part stereotype may be observed in mentally challenged persons. Organic and epileptic patients also give a stereotyped protocol. In psychotic patients, stereotype may give clues to the content of delusion.

Vague Responses: Rorschach inkblots have the potentials for evoking many defined shape objects. If a subject indulges more in producing unstructured or accidental shape objects it is an indication of his free-floating anxiety, this becomes more characteristic if these responses contain any of the shading as the determinant. If on the other hand such vague responses are determined by chromatic color then they would be indicative of impulsivity and a failure to regulate emotional expression.

Variability: Variability is generally found in schizophrenic patients.

Word Finding Difficulty: A difficulty in finding the right word for the perceived object may indicate the process of cognitive deterioration. It has to be ruled out that the person is failing because of marked anxiety.

CHAPTER-10: DIAGNOSTIC INDICATORS

Diagnostic Indicators:

1. In this chapter, diagnostic clustering of indices are presented for reference purposes.
2. It should be remembered that no blind diagnosis is made just on the basis of the clustering of diagnostic indices mentioned here.
3. The diagnosis involves thorough understanding of psychopathologies and their manifestation. Considerable supervised training is required to arrive at a diagnosis through Rorschach Test.
4. You may be surprised to find abnormal findings in apparently healthy individuals.
5. There is no guideline as how many indices from a group should be present to make a diagnosis.
6. A diagnosis may be arrived at on the basis of combined findings in case history, mental status examination and psychological tests.
7. It is recommended that a beginner should administer, score and interpret the Rorschach on clear cases of psychiatric disorders. One should try to work out at least five cases of each common disorders to appreciate the clustering of indices.

Organicity (Brain Pathology):

1. Total number of responses less than 15
2. Response time for a response is greater than 60 seconds
3. Not more than one human action (M)
4. Low F+%
5. Low P
6. FC not more than one
7. Pure color or Color-Form responses

8. Color naming
9. Perseveration
10. Impotence (Awareness of Inadequate Percepts)
11. Automatic phrases
12. Perplexity
13. Enumeration
14. Word finding difficulty
15. Repetition of the same response to the same card on same blot area
16. Confabulation
17. PO responses
18. Failure in recalling one's responses in enquiry phase
19. Mono-response protocol

Psychosis:

1. Low total number of responses
2. Low shape appropriate responses ($F^{+}\%$)
3. Low most frequent responses (P)
4. Confused sequence
5. Color naming
6. Perseveration
7. Variable RT
8. Confabulation
9. Contamination
10. Multiple card rejection
11. PO responses
12. Stereotype
13. P failure on Card-V
14. Bizarre responses
15. Personal reference

Schizophrenia:

1. Low shape appropriate responses (F+%)
2. High CF responses
3. Absence of or low most frequent responses (P)
4. Ancient objects
5. P failure on Card-V
6. Multiple card rejection
7. Multiple response rejection
8. Confabulation
9. Contamination
10. Bizarre responses
11. Confused sequence
12. PO responses
13. Card description
14. Color naming
15. Variable RT
16. Disease percepts
17. Perseveration
18. Stereotype
19. Personal reference
20. Many sex responses
21. Edging (Odd Card Position)
22. Mono-response protocol
23. Non-productive handling of cards
24. Variability

Paranoid Schizophrenia:

1. High W or Dd
2. Very high human action (M) responses
3. Few chromatic color responses
4. High White Background Areas (S%)
5. High human content
6. High part objects like teeth, eyes
7. Looking at the back of the card
8. Hidden percepts
9. Pseudo-human percepts
10. Multiple card rejection
11. Complete rejection
12. Personal reference

Mania:

1. Very high total number of responses
2. High human action (M)
3. Quick RT₁
4. High color responses
5. High Black & White (Y) responses
6. Extratensive EB
7. High affective ratio (more than .80)
8. Flower content
9. Wide content range
10. High religious content
11. More and rapid responses to color blot areas
12. New and additional responses in enquiry
13. Response replacement

Depression:

1. Delayed RT_1
2. Low total number of responses
3. Low or Zero White Background Areas (S%)
4. High Black & White (Y) responses
5. High internal organs (An)
6. Low human action (M)
7. High animal responses (A%)
8. Low total blot area (W)
9. Narrow content range
10. Qualifiers
11. Tendency to reject cards
12. Difficult enquiry
13. Pathological anatomy

Neurosis:

1. Average or above average $F^{+}\%$
2. Average most frequent responses
3. Black & White (Y) response
4. 3 Dimension (V) responses
5. Tactile (T) responses
6. Qualifiers
7. Regular, Irregular or Methodical sequence
8. Color shock or shading shock
9. X responses

Obsessive-compulsive Disorder:

1. High total number of responses
2. Dd dominated approach
3. High human action (M)
4. High White Background Areas (S%)
5. Presence of V, Y, T
6. High F+%
7. Low C or CF
8. Average or high most frequent responses (P)
9. Ambiequal EB
10. Emphasis on symmetry
11. Methodical sequence
12. Denial of chromatic color
13. x responses
14. Expression of doubts

Anxiety Disorder:

1. Low total number of responses
2. Presence of V,Y,T responses
3. High Dd responses
4. Average F+%
5. Qualifiers
6. Frequent and rapid card turning
7. Shading shock or color shock
8. Average most frequent responses (P)
9. Card rejection – IX
10. Vague responses
11. x responses

Hysteria:

1. Shape inappropriate internal organ responses high
2. High sex responses
3. Low total number of responses
4. Extratensive EB
5. Low human content
6. Low human action (M)
7. High Color-Form (CF) responses
8. Card rejections – VI, VII, IX
9. Card descriptions

Epilepsy:

1. Too low A%
2. High R
3. Delayed RT₁
4. Low F+%
5. M- responses
6. Pure C responses
7. Extratensive EB
8. Po responses
9. Confabulation
10. Confused sequence
11. Perseveration
12. Stereotype
13. Many sexual responses

CHAPTER -11: REPORT WRITING

1. A format for report writing is provided here. Instead of adopting the report format you may also generate a report in paragraphs interpreting various variables. In the beginning you may find the format much easier.
2. The reference values in the report format for children are derived from the work of Jain (2002) and for adults from the work of Kumar (2002) and Mahapatra (2007). Instead of these reference values, the normative data of Beck et al. (1961) or the norms developed by Indian researchers published in Pershad and Parekh (2001) may also be used.
3. The reference value in the report formats is the average value for TR and mean percent value for remaining indices.
4. In the results column you will be required to enter the values computed for the protocol of a subject.
5. In rating column you will be required to indicate whether the results are low, average or high in comparison to reference value. There are no guidelines to rate a result as high, average or low. However, you may have a framework of about 10%. That is, if the result exceeds by 10% of the reference value, rate it as high, if the result is less than 10% of the reference value rate it as low and if the result is within $\pm 10\%$ rate it as average.
6. In the interpretation column, you will be required to mention appropriate interpretation of the results.
7. For qualitative indices you may either mention only the name of the indices present in a protocol or you may add the interpretation of the qualitative indices present in the protocol.
8. If there is less space in the table provided in the report format you may expand it and print the report on a larger paper or in landscape orientation.
9. In the clinical summary point, mention the positive findings of case history, mental status examination and information available from any other sources.
10. In the impression column mention the diagnosis appropriate to the test results and clinical information.

11. If you are not confident in arriving at a diagnosis from the available data, you may repeat the Rorschach administration.
12. If you are not sure of a diagnosis you could mention the most probable diagnosis as Under Observation.
13. If appropriate you may add differential diagnosis too in the report.
14. You may administer other tests along with Rorschach, in such cases you will be required to present the results of each tests administered and integrate the Rorschach results in the aggregate report.
15. For arriving at a diagnosis consider all the information and data available from case history, mental status examination, Rorschach, other psychological tests and reports of other investigations like EEG, CT, MRI.
16. If there is any data that goes against the diagnosis you made, you may mention it in the Remarks.
17. Always remember that the diagnosis generated through psychological test results is only suggestive of a diagnosis. Rorschach results are not confirmatory of any diagnosis.

RORSCHACH PSYCHODIAGNOSTIK REPORT (Modified Beck's Scoring System)

Patient's Name:

Date:

Reg. No.

Gender:

Age:

Variable	Symbol	Reference value (Adults)	Result	Rating	Interpretation
Total Number of Responses	TR	22			Productivity
Total Blot Area	W%	20%			Organizing and synthesizing Capacity
Common Blot Area	D%	77%			Practical Approach
Uncommon Blot Area	Dd%	03%			Emphasis on Minute Details
White Blot Area	S%	05%			Opposition Tendencies
Shape Appropriate	F+%	76%			Reality Testing
Action	M%	07%			Imaginative Processes
Color & Color-Form	C+CF%	05%			Impulsivity
Form-Color	FC%	02%			Matured Emotionality
Black & White	Y%	01%			Feelings of Sadness
Tactile	T%	00			Apprehensions/ Anxiety
3Dimension	V%	02%			Inferiority Feelings
Animal	A%	53%			Awareness of Environment
Human	H%	17%			Capacity to Form and Maintain Human Relationships
Internal Organs	An%	04%			Somatic concern
Most Common Responses	P%	18%			Conformity to Social Norms
Qualitative Signs					

Clinical Summary:

Impression:

Remarks:

(Clinical Psychologist)

RORSCHACH PSYCHODIAGNOSTIK REPORT

(Modified Beck's Scoring System)

Patient's Name:

Date:

Reg. No.

Gender:

Age:

Variable	Symbol	Reference value (Children)	Result	Rating	Interpretation
Total Number of Responses	TR	21			Productivity
Total Blot Area	W%	13%			Organizing and synthesizing Capacity
Common Blot Area	D%	77%			Practical Approach
Uncommon Blot Area	Dd%	10%			Emphasis on Minute Details
White Blot Area	S%	01%			Opposition Tendencies
Shape Appropriate	F+%	55%			Reality Testing
Action	M%	06%			Imaginative Processes
Color & Color-Form	C+CF %	03%			Impulsivity
Form-Color	FC%	02%			Matured Emotionality
Black & White	Y%	05%			Feelings of Sadness
Tactile	T%	00%			Apprehensions/ Anxiety
3Dimension	V%	03%			Inferiority Feelings
Animal	A%	51%			Awareness of Environment
Human	H%	18%			Capacity to Form and Maintain Human Relationships
Internal Organs	An%	05%			Somatic concern
Most Common Responses	P%	14%			Conformity to Social Norms
Qualitative Signs					

Clinical Summary:

**Impression:
Remarks**

(Clinical Psychologist)

CHAPTER-12: RESPONSES FOR PRACTICE SCORING

Plate-I	
	R: A flying bat
	E: wings, middle portion, it flies in the night, it is black
	R: A lady
	E: a lady with raised hands, looks like a lady
	R: A fish
	E: looks like a fish because of its shape
	R: Windows
	E: looks like windows you see the depth inside it
	R: Bell
	E: shape of a bell

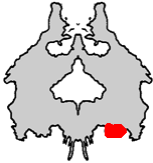
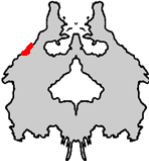
Plate-II	
	R: Two dancing bears
	E: shape is like bears, they are black
	R: Cap
	E: shape is just like a cap, it is red
	R: Shivalinga
	E: shape and black color
	R: Mouth
	E: animal's mouth, because of shape
	R: Blood
	E: only because of red color

Plate-III	
	R: Squirrel
	E: tail is up, sitting position
	R: Bowtie
	E: shape and color
	R: Human face
	E: looks like a face, bent backward slightly
	R: Gun
	E: because of shape
	R: Pelvis
	E: shape

Plate-IV

 TBA	R: Tree
	E: only because D1 is a tree trunk
	R: Black shoe
	E: shape
	R: Fish
	E: shape
	R: Hill
	E: slope, shape
	R: Face
	E: animal's face Shape

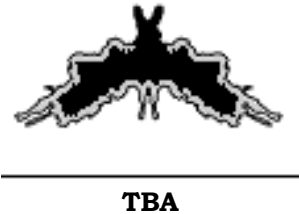
Plate-V	
	R: A bat flying in the sky
	E: the surrounding white area is the sky
	R: A human being
	E: appears to be an angry person
	R: Bird
	E: Shape
	R: Mountain
	E: black color, up and down pattern
	R: Legs
	E: bird's legs, shape

Plate-VI	
	R: Animal Skin
	E: It is very furry and very soft, also it is grayish
	R: Flower
	E: petals, just looks like a flower
	R: Face
	E: it is a human face. Looks like because of shape
	R: Snake
	E: shape, as if moving
	R: Canal
	E: there is a passage, going in the middle

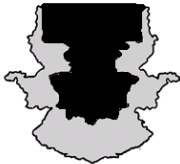
Plate-VII	
	R: Human face
	E: hairs are up, facial features, as if thinking something
	R: Fist
	E: shape
	R: Butterfly
	E: it is black, wings are spread
	R: Flower Vase
	E: shape
	R: Thumb
	E: shape

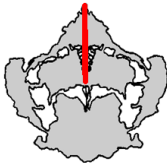
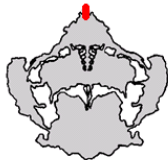
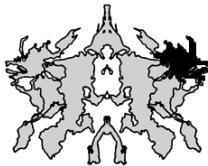
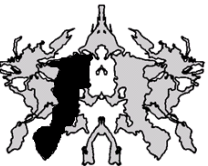
Plate-VIII	
	R: Tree
	E: colored leafs, branches
	R: Beautiful butterfly
	E: looks like a butterfly, wings, body, color
	R: Mountain
	E: slope, shape
	R: Stick
	E: shape
	R: Tooth
	E: shape, human tooth

Plate-IX	
	R: An insect
	E: looks like an insect, colored insect
	R: Cat
	E: because of shape
	R: Light bulb
	E: shape, yellowish light is spreading
	R: Face
	E: it is a face of a baby
	R: Tree
	E: shape

Plate-X	
	R: Crab
	E: Shape
	R: ↓ Human
	E: in standing position
	R: Blood spot
	E: it is reddish
	R: Lion
	E: shape, it is yellow
 TBA	R: Flower garden
	E: colored trees and flowers

KEY					
Plate-I	W Y+ A P	D M+ H P	D F- A	Dds V+ Arc	Dd F+ Ms
Plate-II	D M.Y+ A P	D FC+ Cg	D Y+ Rl	Dd F- Ad	Dd C+ Bl
Plate-III	D F+ A	D FC+ Cg P	D M+ Hdx	D F- Ms	D F+ An
Plate-IV	DW F- Bt	D Y+ Ms P	D F- A	Dd V+ Ls	Dd F- Ad
Plate-V	Ws F+ A P	D M+ H	D F- A	D Y.V+ Ls	D F+ Adx
Plate-VI	D T.V+ Ad P	D F+ Fl	D F- Hd	D F+ A	D V+ Ms
Plate-VII	D M+ Hd P	D M+ Hd	D Y+ A	Ds F+ Ms	Dd F+ Hd
Plate-VIII	D FC+ Bt	D FC+ A	D V+ Ls	Dd F+ Ms	Dd F- Hd
Plate-IX	D FC- A	D F- A	D FC+ Ms	D F+ Hd P	D F- Bt
Plate-X	D F+ A P	D M+ H	D CF+ Bl	D FC+ A P	W CF+ Bt

CHAPTER- 13: EXCERCISES IN QUALITATIVE SCORING

1. Total Blot Area (W) which has been elaborated upon an Uncommon Blot Area (Dd).
2. A Rorschach protocol which contains zero Human Action (M) responses
3. Fusion of two shape appropriate (F+) objects into a single response yielding an inappropriate combination.
4. Repetition of a single response to every inkblot
5. Pointing out one's weakness and deficiencies
6. Turning the card to the backside and reading publisher's details
7. Rotation of several cards 4-5 times before responding
8. Returning a card without producing any response after looking at it for several minutes
9. Denying to have seen an object in the enquiry phase
10. Production of new associations during enquiry
11. Too many internal organ (An) responses in a protocol (more than 60%)
12. Telling colors like red, yellow, blue
13. Not able to provide details of the perceived object in the enquiry phase
14. P failure, decreased F+, low R, delayed RT₁ on a card compared to the pattern on other cards
15. Zero popular responses on Plate-V
16. No scorable associations on four cards
17. Counting such as 1, 2, 3, 4...
18. Blot Area Sequence – W Dd D; D W Dd; Dd W D on three cards
19. Providing details of the structure of the inkblot
20. Viewing a card at odd card positions
21. The responses like fever, cancer, HIV

22. Perception of an ill organ
23. Viewing personal objects like my dog
24. The pattern of RT_1 : 10", 74", 20", 14", 94", 5"
25. More than 60" RT_1 on maximum cards.
26. The responses like clouds, trees, smoke, fire
27. Not able to recall name of the perceived object
28. Perception of an object because of its unique location
29. Zero Most Frequent Responses (P) in a protocol
30. A flying elephant
31. Zero scorable responses in a protocol
32. Not able to improve a poor percept
33. Only two object categories for 20 scorable responses
34. Zero chromatic colors in a protocol of 30 scorable responses
35. Attribution of Human Action (M) to an animal precept
36. Pointing out similarities in both sides of an inkblot

Qualitative Scoring Key

Sl. No.	Answer	Sl. No.	Answer
1	Confabulation	19	Card Description
2	Absence of M Responses	20	Odd Card Position (Edging)
3	Contamination	21	Disease Percept
4	Mon-response Protocol	22	Pathological Anatomy
5	Qualifiers	23	Personal Reference
6	Looking at the Back of the Card	24	Variable RT ₁
7	Excessive Card Turning	25	Delayed RT ₁
8	Non-productive Handling of a Card	26	Vague Responses
9	Response Rejection	27	Word Finding Difficulty
10	New Responses in Enquiry	28	PO Response
11	Anatomy Stereotype	29	Absence of Popular Responses
12	Color Naming	30	Bizarre Response
13	Difficult Enquiry	31	Complete Rejection
14	Shock	32	Impotence (Awareness of Inadequate Percepts)
15	P Failure on Card-V	33	Narrow Content Range
16	Multiple Card Rejection	34	Absence of Color Responses
17	Enumeration	35	M in Animal Percept
18	Confused Sequence	36	Emphasis on Symmetry

CHAPTER-14: MULTIPLE CHOICE QUESTIONS (MCQ)

1. Hermann Rorschach was a Swiss:
 - a. Psychiatrist
 - b. Neurologist
 - c. Philosopher
 - d. Clinical Psychologist
2. Rorschach Inkblots consists of ---- polychromatic images:
 - a. 4
 - b. 3
 - c. 6
 - d. 10
3. Rorschach Inkblot Test is a:
 - a. Structured test
 - b. Semi-structured test
 - c. Unstructured test
 - d. Objective test
4. Rorschach is a test of intelligence:
 - a. True
 - b. False
 - c. Uncertain
5. Color shock is a neurotic phenomenon:
 - a. True
 - b. False
 - c. Uncertain
6. Who developed Indian norms for Rorschach Inkblot Test?
 - a. Prof. M. P. Gulati
 - b. Prof. M. Harshwardhan
 - c. Prof. H. S. Asthana
 - d. Prof. D.K. Awasthi
7. Who authored a book on Rorschach Inkblot Test in India?
 - a. Prof. Dwarka Pershad
 - b. Prof. B.L. Dubey
 - c. Dr. R. K. Mishra
 - d. All of the above

8. Which Rorschach plate evokes an attitude towards male authority?
 - a. IV
 - b. V
 - c. VI
 - d. VII
9. Which of the following indices can be observed in a schizophrenic patient?
 - a. Perseveration
 - b. Color Naming
 - c. Multiple Card Rejection
 - d. All of the above
10. Which of the following indices can be observed in an OCD patient?
 - a. Dd dominated approach
 - b. Contamination
 - c. Perseveration
 - d. All of the above
11. Who developed Rorschach scoring system?
 - a. J.P. Guilford
 - b. Wilhelm Wundt
 - c. M.P. Holtzman
 - d. None of the above
12. M:Csum is a measure of:
 - a. Experience actual
 - b. Experience balance
 - c. Motor Control
 - d. G factor
13. Popular responses indicate:
 - a. Stereotyped thinking
 - b. Irritability
 - c. Group conformity
 - d. All of the above
14. Creativity is tapped by which of the following Rorschach indices?
 - a. White Background Areas (S)

- b. 3 Dimension Responses (V)
 - c. Human Action Responses (M)
 - d. None of the above
15. Concrete thinking is tapped by which of the following Rorschach indices:
- a. Common Blot Areas (D)
 - b. Color-Form Responses (CF)
 - c. Affective Ratio (Afr)
 - d. All of the above
16. White Background Areas (S%) is suggestive of:
- a. Negativistic, oppositional and suspicious behavior
 - b. Obedient, compliant and submissive behavior
 - c. Reality Testing
 - d. None of the above
17. Which of the following indices indicates anxiety?
- a. Excessive card turning
 - b. Shading shock
 - c. Tactile (T) responses
 - d. All of the above
18. Which of the following indices can be observed in a manic patient?
- a. High affective ratio
 - b. High color responses
 - c. High flower content
 - d. All of the above
19. Introversive experience balance comprises of:
- a. High human action (M)
 - b. High color responses
 - c. Both
20. Which of the following indices can be observed in a paranoid patient?
- a. Eye responses
 - b. Hidden percepts
 - c. Pseudo-human percepts
 - d. All of the above

21. Rorschach test results are sufficient enough to diagnose a psychiatric disorder!
- a. True
 - b. False
 - c. Uncertain
22. Rorschach is a highly reliable and valid tool!
- a. True
 - b. False
 - c. Uncertain
23. The scoring and interpretation of Rorschach test require supervised training!
- a. True
 - b. False
 - c. Uncertain
24. Mono-response protocol can be observed in which of the following psychiatric disorders?
- a. Schizophrenia
 - b. Brain pathology
 - c. Mental retardation
 - d. All of the above
25. Rorschach is a group test!
- a. True
 - b. False
 - c. Uncertain
26. Rorschach test can not be administered on children!
- a. True
 - b. False
 - c. Uncertain
27. The object category for 'human face' is:
- a. H
 - b. (H)
 - c. Hd
 - d. (Hd)
28. FM is scored for following animal movement in Beck's scoring method
- a. Flying bat

- b. Barking dog
 - c. Both (a) and (b)
 - d. None of the above
29. Which of the following indices indicate matured emotionality?
- a. Color-Form (CF)
 - b. Form-Color (FC)
 - c. Lambda (L)
 - d. Response Time
30. Card positions are recorded during response phase of the Rorschach test administration!
- a. True
 - b. False
 - c. Uncertain
31. Rapport with the subject is not required prior to Rorschach administration!
- a. True
 - b. False
 - c. Uncertain
32. In Beck's method, enquiry can be conducted immediately after obtaining responses on a card!
- a. True
 - b. False
 - c. Uncertain
33. Reality testing on Rorschach is taped by which of the following indices?
- a. Shape appropriate responses (F+%)
 - b. PO responses
 - c. Total Blot Area (W)
 - d. All of the above
34. Many new responses in enquiry can be observed in which of the following psychiatric disorder?
- a. Dementia
 - b. Bipolar Disorder
 - c. Depression
 - d. OCD

35. Rorschach Inkblot Test is published by
- Psychological corporation
 - Grune & Stratton
 - Rorschach Society
 - Verlag Hans Huber
36. Which of the following researchers worked on Rorschach Inkblot Test for Doctoral Thesis?
- Deepak Kumar
 - Ruchi Jain
 - Jashobanta Mahapatra
 - All of the above
37. Rorschach protocols can be summarized and interpreted through computerized software!
- True
 - False
 - Uncertain
38. Which of the following is the scoring for Blot Areas?
- W, D, Dd, S
 - F, M, C, V, Y, T
 - H, Hd, A, Ad, An
 - Afr, Csum, EB
39. Who conducts training workshops on Rorschach Inkblot Test in India?
- Prof. Dwarka Pershad
 - Dr. Rakesh Jain
 - Dr. Kakli Gupta
 - All of the above
40. Which of the following journals publishes maximum number of researches on inkblots in India?
- Indian Journal of Clinical Psychology
 - Indian Journal of Mental Health and Disabilities
 - Indian Journal of Social Science Researches
 - SIS Journal of Projective Psychology and Mental Health
41. Which of the following associations/Society conducts International Conferences on Inkblots in India?
- Indian Association of Clinical Psychologists

- b. Jewels International
 - c. Indian Psychological Association
 - d. Somatic Inkblot Society
42. In which year Rorschach Inkblot Test was published?
- a. 1879
 - b. 1910
 - c. 1921
 - d. 1922
43. Which of the following is not a projective test?
- a. SIS
 - b. TAT
 - c. MMPI
 - d. DAPT
44. Which of the following indices can be observed in an organic patient?
- a. Perplexity, Impotence, Perseveration, Automatic Phrases
 - b. High M, High FC, High R and High P
 - c. Both (a) and (b)
 - d. None of the above
45. Repetition of the same response to three or more consecutive cards without any intervening response is called?
- a. Perseveration
 - b. Contamination
 - c. Confabulation
 - d. None of the above
46. A running dog would be scored as M determinant
- a. True
 - b. False
 - c. Uncertain
47. Rorschach test can not be administered on illiterate persons!
- a. True
 - b. False
 - c. Uncertain
48. Which of the following is not a Rorschach scoring method?
- a. Selva Method

- b. Beck's Method
 - c. Exner's Method
 - d. Klopfer's Method
49. Which of the following plates elicits a W response less frequently?
- a. I
 - b. IV
 - c. V
 - d. X
50. What is the title of the book authored by Hermann Rorschach?
- a. Rorschach Scoring Method
 - b. Rorschach Inkblot Test
 - c. Scoring and Interpretation of Inkblots Associations
 - d. Rorschach Psychodiagnostik

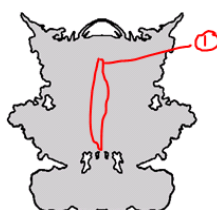
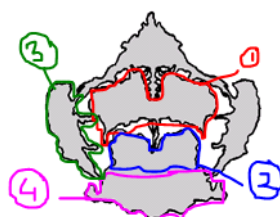
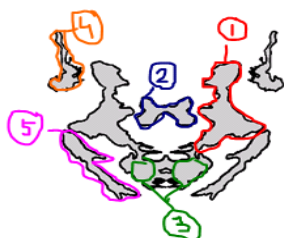
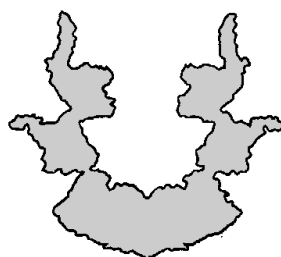
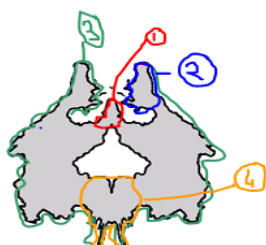
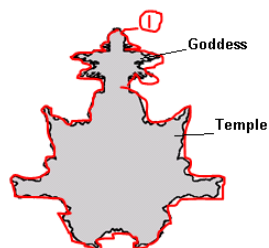
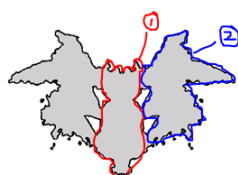
Multiple Choice Questions (MCQ) Key									
1	2	3	4	5	6	7	8	9	10
a	b	c	b	a	c	d	a	d	a
11	12	13	14	15	16	17	18	19	20
d	b	c	c	a	a	d	d	a	d
21	22	23	24	25	26	27	28	29	30
b	b	a	d	b	b	c	d	b	a
31	32	33	34	35	36	37	38	39	40
b	b	a	b	d	d	a	a	d	d
41	42	43	44	45	46	47	48	49	50
d	c	c	a	a	b	b	a	d	d

CHAPTER-15: CASE STUDIES

Case-1: Response Sheet					
Name of the Subject: GM			Subject ID: x		
Date of Administration: 20/7/09			Education: 5th		Age: 40 years
C.N*	RT ₁	RT _T	CP	Responses	Enquiry
I	7"	35"	↑ ↑	1. Gas Cylinder 2. Bird	1. Shape 2. wings, face, body (New R: Bear)
II	11"	67"	↑ ↑ ↑ ↑	1. Temple 2. Bird 3. Boy and Girl 4. Red drops	1. Temple peak, shape 2. Shape 3. Dancing 4. --
III	6"	58"	↑ ↑ ↑ ↓ ↑	1. Man 2. Dumroo 3. Testicles 4. Monkey 5. Airplane	1. Bent forward 2. Shape 3. Shape 4. Shape 5. Shape
IV	7"	24"	↑	1. Demon	1. Big legs, hairs in the middle, small hand, looking straight
V	5"	17"	↑	1. Bat	1. Shape
VI	8"	22"	↑	1. A goddess sitting on temple	1. looks like a goddess and a temple
VII	16"	29"	↑	1. A female	1. Response Rejection (New R: Spinal Cord)
VIII	7"	78"	↑ ↑ ↑ ↑	1. Flag 2. Red teared clothe 3. Monkeys 4. Earth	1. Colored flag 2. Shape 3. Climbing 4. Earth is painted yellow
IX	6"	38"	↑ ↑	1. Stick 2. Red Color, Green Color	1. Colored stick (New R: Bulb) 2. Color Naming
X	6"	84"	↑ ↑ ↑ ↑ ↑	1. Spinal Cord 2. Green Flowers 3. Green Flag 4. Red Legs 5. Bird	1. Shape 2. Looks like flowers because of green color 3. Looks like a flag 4. Animal legs (New R: Insect) 5. Shape

CASE-1: Location Chart

Name: Mr. GM. Date: 20/7/2009



Case-I: Scoring

Mr. G.M. Date: 20/7/2009

Plate-I	D4 F- Ms	D2 F+ A	-	-	-
Plate-II	D4 V+ Rl	D2 F+ A	W M+ H P	D3 CF+ Ms	-
Plate-III	D8 M+ H	D3 F+ Ms	D10 F- Sx	D2 F+ A	D6 F+ Tr
Plate-IV	W M+ H	-	-	-	-
Plate-V	W F+ A P	-	-	-	-
Plate-VI	W M.V+ H, Rl	A compound Response. Single W scoring is more appropriate			
Plate-VII	-	-	-	-	-
Plate-VIII	D5 CF+ Ms	D6 CF- Cg	D1 F+ A	D7 CF- Ms	-
Plate-IX	D4 FC+ Ms	-	-	-	-
Plate-X	D14 F+ An	D4 CF- Fl	D12 FC- Ms	D5 FC- Ad	D2 F+ A
Qualitative: New Responses in Enquiry; Response Rejection, Color Naming, Wide Content Range					

RORSCHACH PSYCHODIAGNOSTIK REPORT

(Modified Beck's Scoring System)

Patient's Name: Case-1: GM

Date: 20/7/2009

Reg. No. x

Gender: Male

Age: 40 Years

Variable	Symbol	Ref. Value (Adults)	Results	Rating	Interpretation
Total Number of Responses	TR	22	24	Average	Productivity
Total Blot Area	W%	20%	17	Average	Organizing and synthesizing Capacity
Common Blot Area	D%	77%	83	Average	Practical Approach
Uncommon Blot Area	Dd%	03%	00	Low	Emphasis on Minute Details
White Blot Area	S%	05%	00	Low	Opposition Tendencies
Shape Appropriate	F+%	76%	82	Average	Reality Testing
Action	M%	07%	17	High	Imaginative Processes
Color & Color-Form	C+CF%	05%	21%	High	Impulsivity
Form-Color	FC%	02%	13	High	Matured Emotionality
Black & White	Y%	01%	00	Average	--
Tactile	T%	00	00	Average	--
3Dimension	V%	02%	08	High	Inferiority Feelings
Animal	A%	53%	29	Low	--
Human	H%	17%	17	Average	Capacity to Form and Maintain Human Relationships
Internal Organs	An%	04%	04	Average	Somatic concern
Most Common Responses	P%	18%	08	Low	Conformity to Social Norms
Qualitative Signs	New Responses in Enquiry; Response Rejection, Color Naming, Wide Content Range, M.V Response, Extratensive EB				

Clinical Summary: The patient presented with the second episode of psychiatric illness of one month duration presently characterized by increased psychomotor activity, volubility, grandiosity and elated mood.

Impression: **Bipolar Disorder**

Remarks:

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- sd -

(Clinical Psychologist)

Case-2: Response Sheet

Name of the Subject: AKC

Subject ID: X

Date of Administration: 29/7/200

Education: B.A.

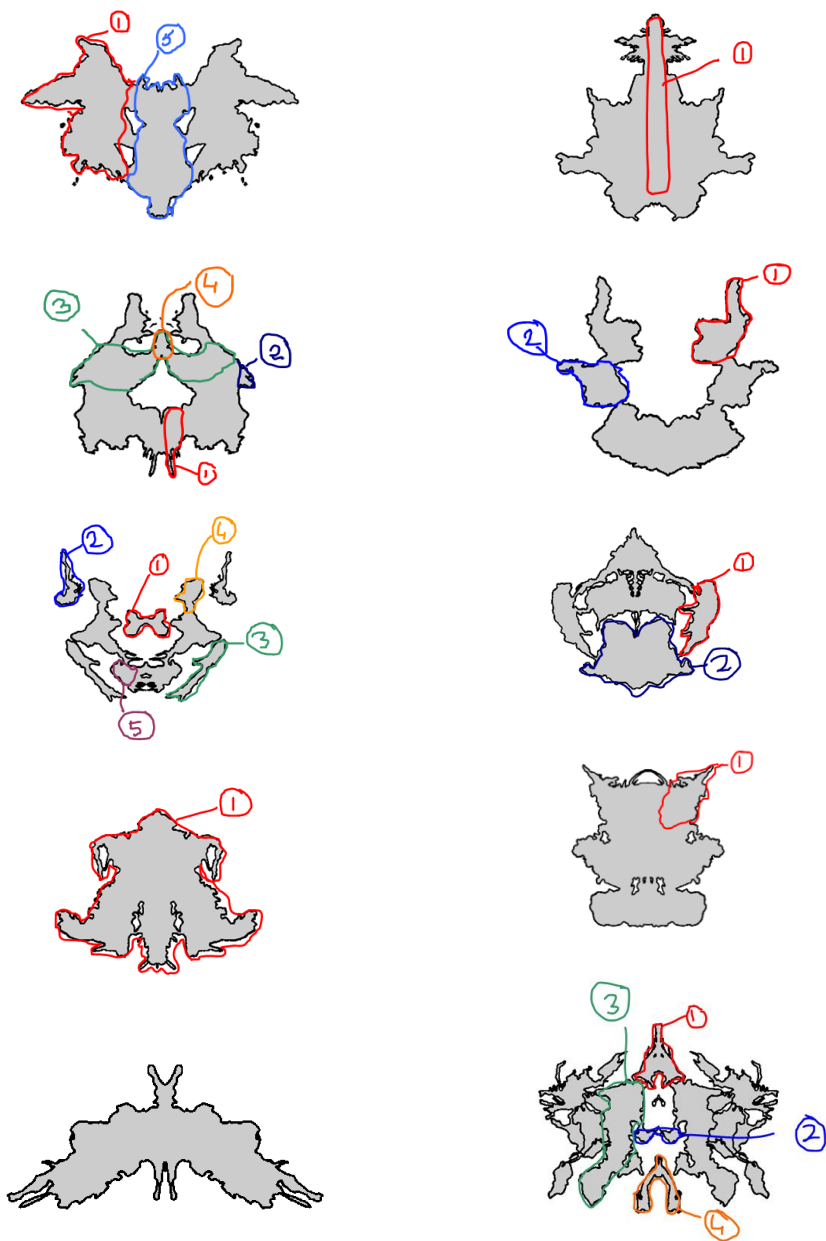
Age: 29 years

C.N*	RT ₁	RT _T	CP	Responses	Enquiry
I	Recording of RT May be Skipped		↑ ↑ ↑ ↑ ↑	1. Stone 2. Pictures 3. Ancient Statue 4. Mountain 5. Statue	1. Zigzag shape, up and down, lotus type dome, mountains in both sides, statute in middle, black mountain, tower 2. Rejection 3. Rejection 4. Rejection 5. Legs, nose
II			↑ ↑ ↑ ↑ ↑	1. Snake 2. Frog 3. Pictures on Stones 4. Dome 5. Stone Carving	1. Shape 2. Shape, eyes, mouth 3. White stone, shape 4. Dome above a mosque 5. Rejection
III			↑ ↑ ↑ ↑ ↑	1. Dumbbell 2. Two Rats 3. Fish 4. Camel's Face 5. Elephant	1. Shape, place to hold in the middle 2. Mouth, legs, eyes, tail 3. Big size, wings, mouth 4. Mouth, neck 5. Shape (New R: rabbit)
IV			↑	1. Variety of Ancient Statues	1. Eyes, horns of deities
V			↑ ↑	1. Crocodile 2. Strange Creature	1. Rejection 2. Rejection
VI			↑	1. Spinal Cord, Nervous System from Brain to Spine base	1. Eyes, trunk, brain spine

VII		↑ ↑ ↑ ↑	1. Lady's face 2. Ancient man's face 3. Face of a chimpanzee 4. Monkey's face	1. Mouth, nose, eyebrows, hairs 2. Jaws, nose, front portion 3. Rejection 4. Rejection
VIII		↑ ↑	1. Two monkeys 2. Rocks	1. Mouth, legs, tail 2. Colored rocks
IX		↑ ↑ ↑	1. Singing Dancing Drawing 2. Human being dancing in artificial costume 3. Scenery	1. Human being, shape 2. Rejection 3. Rejection
X		↑ ↑ ↑ ↑ ↑	1. Nervous system 2. Pair of kidney 3. Lungs 4. Urinary bladder 5. Body Parts	1. Shape 2. Shape 3. Shape 4. Shape 5. Rejection

CASE-2: Location Chart

Mr. AKC Date: 29/7/2009



Case-2: Scoring

Mr. AKC Date: 29/7/2009

Plate-I	D2 V.Y+ Ls	D4 F+ Ms	-	-	-
Plate-II	Dd F+ A	Dd F- A	Dd Y- Ms	D4 V+ Rl, Arc	-
Plate-III	D3 F+ Ms	D2 F- A	D F+ A P	D7 F- Ad	D10 F- A
Plate-IV	W F+ Ms	-	-	-	-
Plate-V	Response Rejection	-	-	-	-
Plate-VI	D4 F+ An	-	-	-	-
Plate-VII	D1 F+ Hd P	D3 F- Hd	-	-	-
Plate-VIII	D1 F+ A	D2 CF+ Ms	-	-	-
Plate-IX	D M+ H P	-	-	-	-
Plate-X	D11 F- An	D F- An	D5 F+ An	D10 F- An	-

Qualitative: Multiple Response Rejection, P-Failure on Card-V, Ancient Objects, Difficult Enquiry,

RORSCHACH PSYCHODIAGNOSTIK REPORT

(Modified Beck's Scoring System)

Patient's Name: Case-II AKC

Date: 29/7/2009

Reg. No. x

Gender: Male

Age: 29 Years

Variable	Symbol	Ref. value (Adults)	Result	Rating	Interpretation
Total Number of Responses	TR	22	22	Average	Productivity
Total Blot Area	W%	20%	05	Low	Organizing and synthesizing Capacity
Common Blot Area	D%	77%	82	Average	Practical Approach
Uncommon Blot Area	Dd%	03%	14	High	Emphasis on Minute Details
White Blot Area	S%	05%	00	-	Opposition Tendencies
Shape Appropriate	F+%	76%	53	Low	Reality Testing
Action	M%	07%	05	Average	Imaginative Processes
Color & Color-Form	C+CF%	05%	05	Average	Impulsivity
Form-Color	FC%	02%	00	-	Matured Emotionality
Black & White	Y%	01%	09	High	Sadness
Tactile	T%	00	00	-	--
3Dimension	V%	02%	09	High	Inferiority Feelings
Animal	A%	53%	27	Low	--
Human	H%	17%	14	Average	Capacity to Form and Maintain Human Relationships
Internal Organs	An%	04%	23	High	Somatic concern
Most Common Responses	P%	18%	14	Average	Conformity to Social Norms
Qualitative Signs	Multiple Response Rejection, P-Failure on Card-V, Ancient Objects, Difficult Enquiry, Irregular Sequence, Wide Content Range				

Clinical Summary: The patient presented with a history of nine years of psychiatric illness presently characterized by delusion of reference, delusion of persecution and delusion of misidentification

Impression: Schizophrenia, Paranoid Type

Remarks: --

- sd -

(Clinical Psychologist)

Case-3: Response Sheet

Name of the Subject: PS

Subject ID: X

Date of Administration: 30/7/200

Education: 4th Std. Age: 27 years

C.N*	RT ₁	RT _T	CP	Responses	Enquiry
I	5"	15"	↑ ↑	1. Bat 2. Scorpion	1. Wings and body 2. Shape without legs
II	5"	7"	↑	1. Lungs	1. Shape
III		26"	↑	Card Rejection	-
IV	21"	39"	↑	1. X-ray, human	1. Response Rejection
V	59"	1'10"	↑	1. X-ray human	1. Shape
VI	-	44"	↑	Card Rejection	-
VII	15"	27"	↑	1. Lungs	1. Response Rejection
VIII	54"	54."	↑	1. Scorpion	1. Response Rejection (New R: Insects)
IX	-	1.11"	↑	1. Card Rejection	-
X	-	1.11"	↑	1. Card Rejection	-

Case-3: Location Chart

Mr. P.S. Date: 30/7/2009

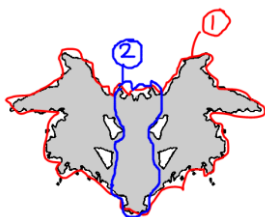


Plate-I

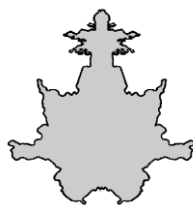


Plate-VI

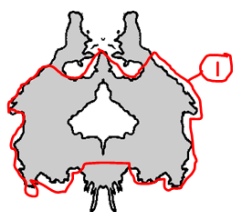


Plate-II

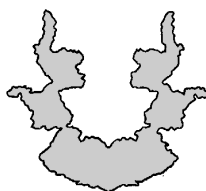


Plate-VII



Plate-III



Plate-VIII



Plate-IV



Plate-IX

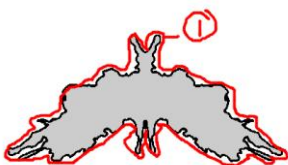


Plate-V



Plate-X

Case-3: Scoring

Mr. PS Date: 30/7/2009

Plate-I	W F+ A P	D4 F+ Ad	-	-	-
Plate-II	D5 F- An	-	-	-	-
Plate-III	-	-	-	-	-
Plate-IV	-	-	-	-	-
Plate-V	W F- An	-	-	-	-
Plate-VI	-	-	-	-	-
Plate-VII	-	-	-	-	-
Plate-VIII	-	-	-	-	-
Plate-IX	-	-	-	-	-
Plate-X	-	-	-	-	-

Qualitative: Card Rejection, Response Rejection, Variable RT, Non-productive handling of cards, P-Failure on Card-V, Multiple Card Rejections, One New Response in enquiry

RORSCHACH PSYCHODIAGNOSTIK REPORT
(Modified Beck's Scoring System)

Patient's Name: Case-3: PS
Gender: Male

Date: 30/7/2009
Age: 27 Years

Reg. No. x

Variable	Symbol	Ref. value (Adults)	Results	Rating	Interpretation
Total Number of Responses	TR	22	04	Very Low	Productivity
Total Blot Area	W%	20%	-	-	-
Common Blot Area	D%	77%	-	-	-
Uncommon Blot Area	Dd%	03%	-	-	-
White Blot Area	S%	05%	-	-	-
Shape Appropriate	F+%	76%	-	-	-
Action	M%	07%	-	-	-
Color & Color-Form	C+CF%	05%	-	-	-
Form-Color	FC%	02%	-	-	-
Black & White	Y%	01%	-	-	-
Tactile	T%	00	-	-	-
3Dimension	V%	02%	-	-	-
Animal	A%	53%	-	-	-
Human	H%	17%	-	-	-
Internal Organs	An%	04%	-	-	-
Most Common Responses	P%	18%	-	-	-
Qualitative Signs	Very Low Productivity (4 R), Response Rejection, Variable RT, Non-productive handling of cards, P-Failure on Card-V, Multiple Card Rejections, One New Response in enquiry, Impaired Reality Testing				

Clinical Summary: The patient presented with six years history of continuous psychiatric illness presently characterized by command auditory hallucinations and delusions of having huge amount of money (Rs. 800 crores)

Impression: **Schizophrenia**

Remarks: --

- sd -

(Clinical Psychologist)

Case-4: Response Sheet

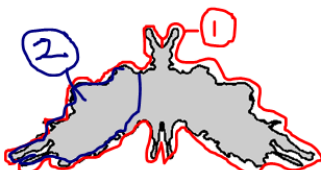
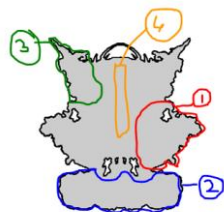
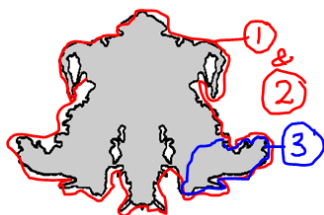
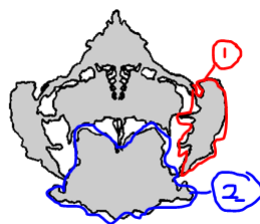
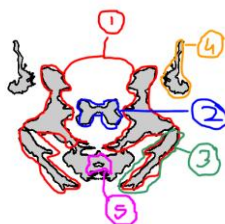
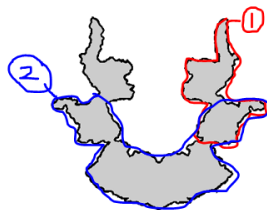
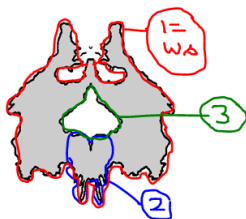
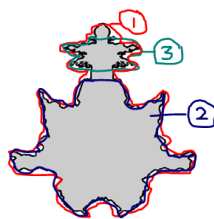
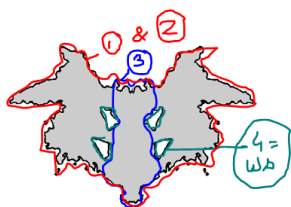
Name of the Subject: Km. Varsha Singh* Subject ID: X
 Date of Administration: 31/7/200 Education: 15th Std.
 Age: 21 years Examiner: Km. Renu Rawat

C.N*	RT ₁	RT _T	CP	Responses	Enquiry
I	5"	32"	↑ ↑ ↑ ↑	1. Bat 2. Butterfly 3. Two persons 4. Mask	1. Structure of Bat 2. Shape 3. Head upside, bottom, legs, hands 4. Shape, eyes and nose
II	5"	33"	↑ ↑ ↑	1. Two human 2. Butterfly 3. Pan ka patta	1. Red cap, face and body 2. Shape, wings and body 3. Shape
III	2"	44"	↑ ↑ ↑ ↑ ↑	1. Two ladies 2. Bowtie 3. Fish 4. Moving horse 4. Insect	1. Shape 2. Shape and color 3. shape, face, wings 4. Legs, looking back 5. Legs, crab type, light gray
IV	9"	1'31"	↑ ↑ ↑	1. Giant 2. Skin 3. Cloud	1. Body structure, thick legs 2. Pressed skin of lion 3. Rainy clouds, gray color
V	3"	36"	↑ ↑	1. Bat 2. Someone lying	1. Color, shape, wings, body 2. Mouth, eyelashes, face, with folded hands
VI	55"	56"	↑ ↓ ↓	1. Skin 2. Two bears 3. sun	1. Face, hands, body parts, animal skin 2. Nose, head, eyes, tongue 3. Half sun, rays
VII	3"	37"	↑ ↑	1. Two woman 2. Two animals	1. shape, forehead, eyes, nose, chin 2. Face, ears, eyes, nose, open mouth
VIII	3"	1'19"	↑ ↑	1. Two lions 2. Hills	1. Shape, four legs, tail, mouth, climbing on mountain

					2. Colored hill, forest like, many trees over hill
IX	23"	2'09"	↑ ↑ ↑ ↑	1. Forest, many trees 2. Newborn baby 3. Two insects 4. Backbone	1. Green color 2. Pink color, small hands, legs 3. Orange insects, hands, strange 3. Long and thin
X	3"	2'57"	↑ ↑ ↑ ↑ ↑	1. Two scorpion 2. Two crabs 3. Two Lions 4. Two animals 5. Two man	1. Many legs, shape 2. Shape and legs 3. Legs, tail, heavy body, mouth up, yellow color 4. Legs, tail, heavy body, ears, horn 5. Cap, face, read dress, hands
* Names used with consent					

Case-4: Location Chart

Km. Varsha Singh. Date: 31/7/2009



Case-4: Scoring

Km. Varsha Singh Date: 31/7/2009

Plate-I	W F+ A P	W F+ A P (Duplicate Response)	D4 M+ H	Ws F+ Ms	-
Plate-II	Ws FC+ H P	D3 F+ A P	Ds6 F+ Fd	-	-
Plate-III	D1 F+ H P	D3 FC+ Ms P	D6 F+ A P	D2 F+ A	D5 Y+ A
Plate-IV	W F+ H	W F+ Ad P	D2 Y+ Cl	-	-
Plate-V	W Y+ A P	D2 M+ H	-	-	-
Plate-VI	W F+ Ad P	D1 F- A	D3 F- Na	-	-
Plate-VII	D2 F+ H P	Dd F+ A	-	-	-
Plate-VIII	D1 F+ A	D2 V. CF+ Ls	-	-	-
Plate-IX	D2 CF- Bt	D3 FC+ H	D1 FC- A	D4 F+ An	-
Plate-X	D1 F+ A	D8 F+ A	D2 FC+ A P	D9 F+ A	D F+ H

Qualitative: Duplicate Response, Quick RT₁

RORSCHACH PSYCHODIAGNOSTIK REPORT

(Modified Beck's Scoring System)

Name: Case-4: Varsha Singh

Date: 31/7/2009

Reg. No. x

Gender: Female

Age: 21 Years

Variable	Symbol	Ref. value (Adults)	Results	Rating	Interpretation
Total Number of Responses	TR	22	32	High	Productivity
Total Blot Area	W%	20%	22	Average	Organizing and synthesizing Capacity
Common Blot Area	D%	77%	75	Average	Practical Approach
Uncommon Blot Area	Dd%	03%	03	Average	Emphasis on Minute Details
White Blot Area	S%	05%	09	Average	Opposition Tendencies
Shape Appropriate	F+%	76%	90	High	Reality Testing
Action	M%	07%	06	Average	Imaginative Processes
Color & Color-Form	C+CF%	05%	06	Average	Impulsivity
Form-Color	FC%	02%	16	High	Matured Emotionality
Black & White	Y%	01%	09	High	Sadness
Tactile	T%	00	00	-	--
3Dimension	V%	02%	03	Average	Inferiority Feelings
Animal	A%	53%	50	Average	--
Human	H%	17%	25	Average	Capacity to Form and Maintain Human Relationships
Internal Organs	An%	04%	03	Average	Somatic concern
Most Common Responses	P%	18%	31	High	Conformity to Social Norms
Qualitative Signs	Duplicate Response, Quick RT ₁				

Clinical Summary: --

Impression: An intelligent, emotionally matured with above average capacity for perceptual clarity and group conformity.

Remarks:

- sd -

(Clinical Psychologist)

CHAPTER-16: BRIEF BIOGRAPHY OF HERMANN RORSCHACH

- Hermann Rorschach (1884-1922) the eldest of four children was born on 8th November 1884 at Zurich.
- Rorschach was nicknamed as Klex (meaning inkblot) derived from the word 'Klecksography'. Klecksography was a popular game in which children used to play by spilling ink over plain paper, fold it and open to view objects in the emerged pattern of inkblot.
- Rorschach's father was a drawing teacher. Like his father, Rorschach also showed a great talent at painting and drawing. He solicited advice from Ernst Haeckel (1834-1919), the famous advocate of Darwin's evolutionary theory, whether he should opt for art or natural sciences for further studies. Haeckel advised to move for natural sciences.
- As per the advice, Rorschach entered medical school in Zurich in 1904 and graduated in 1909.
- Rorschach married to Olga Stempelin, a Russian lady, in 1910. They had a son and a daughter.
- Rorschach accepted a position in an asylum in Münsterlingen in 1909. Rorschach had become very popular in psychiatric patients for his organizations of theatrical programs. At one time he brought a monkey and kept it to observe the patients' reactions to it, and also to entertain them.
- Rorschach was experimenting with inkblots and Jung's word association test on school children and patients in 1911.
- Rorschach had studied under Eugen Bleuler (the eminent psychiatrist who coined the term schizophrenia). Bleuler had supervised his doctoral dissertation which was completed in 1912.
- Towards the end of 1915 Rorschach was appointed associate director of the asylum at Herisau, in the eastern part of Switzerland.
- Rorschach had an intense interest in psychoanalysis. He was also keenly interested in investigating psychopathology

in religious sects in Switzerland. He examined Binggeli, a leader of strange religious sect who taught his disciples that his penis was sacred and that they should adore it; his urine was called 'heaven's drops' or 'heaven's balm' and he gave it to them for the Holy Communion.

- Rorschach's interest in the inkblot test got revoked, when Hens published a doctoral thesis on an inkblot test in 1917. Hens' technique was similar to the one applied by Rorschach in 1911, Hens had studied the fantasies of his subjects using inkblots.
- The work of Hens led Rorschach to resume his own experiments in 1918. He used 15 cards more often than many other cards. He collected the answers to the test from 305 persons, 117 of them non-patients, 188 of them were schizophrenics. He showed them the cards and asked the question: "What might this be?" Their subjective responses enabled him to distinguish among his patients on the basis of their perceptive abilities, intelligence, and emotional characteristics.
- Rorschach's friends were very positive of his work and encouraged him to get it published. The original version of 15 inkblots, were sent to six publishers. All of them refused to publish the work. Eventually a publisher from Bern agreed to publish his work on the condition that the number of cards was reduced to ten.
- The final set of 10 Rorschach Inkblot cards was published in June 1921. But the printing quality of the inkblots was quite different and resulted in many variations. They had been reduced in size, the colors had been altered and the original patches of uniform color density had been reproduced with a varying degree of saturation which later became an important variable (shading).
- His book 'Rorschach Psychodiagnostik' published in 1921 was a disaster. The entire edition remained unsold. The publisher, Bircher, went bankrupt shortly afterwards. Rorschach was somewhat depressed but remained firm in his stand. He delivered a lecture in Swiss Psychoanalytic Society in February 1922, he spoke of further development of his test.
- Rorschach remained in Herisau asylum until his premature death. On 1st April 1922, he was hospitalized after a week of

abdominal pains, probably caused by a ruptured appendix. An explorative laparotomy was performed, but the condition proved to be inoperable, and Rorschach died of peritonitis, the following day, on 2nd April 1922 only at the age of 37 years.

- Hans Huber purchased the publication rights of Rorschach Inkblots from Ernst Bircher in 1927. Since then Hans Huber has been the publisher of Rorschach Psychodiagnostik. The publication of inkblots is still done through the older and obsolete methods to preserve the color, texture and overall Gestalten of the inkblots. The equipments are very carefully and exclusively maintained for publication of inkblots to prevent any distortions.

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President Somatic Inkblot Society
Former President Indian Association of Clinical Psychologists
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Author of ‘Manovikrati Vigyan’



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Ranchi during his M.Phil in Medical and Social Psychology (1987-1989). He is awarded Ph.D. in Psychology by Utkal University, Bhubaneswar in 1997. He is actively engaged in application of Rorschach Inkblot Techniques at Institute of Mental Health and Hospital, Agra since 1998. He has conducted training workshops on Rorschach Inkblots at Utkal University, Bhubaneswar; Dev Sanskriti Viswavidyalaya, Haridwar and Institute of Mental Health and Hospital, Agra. He has individually trained a number of postgraduate students of psychology on Rorschach Inkblot Test. He has also supervised four doctoral theses on Inkblots and two postgraduate research projects. He has published 15 research papers on inkblots. He has significantly contributed in the development of a Comprehensive Scoring System of Somatic Inkblot Series-I. He is the recipient of SIS International Awards in 2004 and 2008 in recognition of major contribution to the advancement of Somatic Inkblot Series.

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